



Booking and Billing Systems Upgrade

A custom scheduling and billing
platform for Anaesthesia Associates

RFP Response - Anaesthesia Associates
Booking and Billing Systems Upgrade

July 2026

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Building a Platform for Reliability and Efficiency: Anaesthesia Associates' Next-Generation Scheduling System

Stratos Technology Partners started 15 years ago with 3 staff. Since then it has grown to 60 people, servicing businesses across New Zealand and Australia. We are still owned and run by the original founders who take great pride in growing a sustainable, values-based business, producing great outcomes for our clients, employees, and community.

We have partnered with a number of healthcare and primary care clients over the years. We have a dedicated team who have developed custom solutions for organisations across the sector, working alongside clinicians, practice managers, and administrators to deliver software that supports greater efficiency and better user outcomes.

We have developed a strong understanding of this sector, including the regulatory environment, building robust integrations with national health systems, and the sensitivities around patient data. Those relationships and that domain knowledge have been key to our continued growth.

In this proposal we have detailed how we will deliver a solution that meets the requirements set out in the RFP, from both a functional and technical perspective. We have included a video of a prototype we built using AI tools. This helps to demonstrate how the use of AI in requirements gathering and rapid prototyping can be extremely valuable in making sure all stakeholders are aligned and requirements are well met and understood.

AI has rapidly become a core tool in software development, and we continually invest in tooling and education in this space. However, it does not replace the human interaction that is crucial to fully understanding any problem and building lasting relationships.

At Stratos, we treat every new project as the beginning of a long-term partnership. Many of our clients have been with us for more than a decade, and through this time we have maintained an excellent reputation working alongside our clients throughout the full technology lifecycle. This looks like providing trusted advice to make informed technology decisions, adapt to changing needs, and ensure systems remain secure, resilient, and delivering value for years to come.

We are excited about the possibility of working with you on this project. We believe our experience in the sector, proven track record of delivering cloud-based web and mobile applications, and our enduring partnership model, mean we will form a fantastic long-term relationship.



David Carter
CEO and Founder
31 July 2026

Our Stratos Vision

To be the technology partner businesses trust, building a 100-year company by doing right by our people, clients, and community, while pioneering innovation in our industry.



Section One

Solution Overview

Solution Overview

The Anaesthesia Associates (AA) Booking and Billing System is essential to business operations. With the current legacy platform at end of life, and no clear off the shelf solutions available, AA requires a modern, purpose-built platform that reduces manual workloads, improves data quality and reliability, and accurately manages complex scheduling and billing requirements.

With our extensive experience building bespoke software in the health sector, we have a strong understanding of the challenges involved in protecting data, integrating with third-party systems, and creating efficient workflows for time-poor users. We will design and build a role-based web and mobile application that supports anaesthetists, administrators, hospitals, and surgeons' rooms to schedule and bill procedures in line with AA's specific rules and processes.

The solution will be delivered as a secure, scalable cloud service, using common, well supported technologies to keep long term costs and operational risk low. We'll design it to be easy to navigate, mobile friendly, with intuitive flows so that users can quickly understand core features.

At its heart the solution is a scheduling application that extends into timesheet capture and billing. Anaesthetists will manage their calendars and capture procedure detail; the office will oversee and approve that work; and a billing engine will turn approved work into invoices. Integrations with booking data and NHI improve robustness and reduce the manual keying of data for greater efficiency. An integration with Xero sits behind the billing engine as an accounts-receivable and banking service rather than as the system of record.

We will adopt AA's candidate architecture and take it as a solid starting point. We concentrate here on the design decisions and mechanisms we would add to it, and on addressing some of the open questions from the RFP. Some questions we have left open to be answered in the discovery phase. All details, intended technologies, and architectural choices are based on our current understanding of the requirements - we will thoroughly validate and revise these details during the discovery phase.



Understanding of Requirements

Anaesthesia Associates provides booking and billing services for approximately 85 independent anaesthetists in Christchurch. Its current platform supports a complex network of recurring and ad hoc theatre bookings, late schedule changes, clinical billing capture, invoicing and payment administration. With many of these activities still requiring manual intervention, AA needs a modern system that improves reliability and data quality, reduces repetitive administration, and gives every user a clear and consistent view of the work requiring their attention.

We propose a unified booking and billing solution comprising three connected application experiences. Anaesthetists will use purpose-built mobile and web applications to view their upcoming AM and PM Lists, understand where and with whom they are working, and complete the patient Cards associated with each operating session. The experience will make it quick to capture procedure details, time and billing information, attach supporting material, add a missing Card when required, and submit a completed List to the office. The wider web experience will also support forward planning, availability, outstanding balances and activity reporting.

The scheduling platform will bring together permanent arrangements, manually advised bookings and updates received from hospitals and surgeons' rooms. Where integrations are available, HL7 and FHIR interfaces will process new bookings, reschedules, amendments and cancellations with minimal re-keying. PDF and photo-assisted capture will provide practical alternatives where automated feeds are not yet available. Patient identity will be validated using NHI information where available, while every material change will be traceable through a comprehensive audit history.

For AA's office team, a dedicated administration application will provide a whole-of-service view across the anaesthetist roster. Staff will be able to manage the rolling schedule, coordinate ad hoc bookings, respond to late changes, move individual Cards, arrange cover by reassigning whole Lists, and resolve integration or data-quality exceptions. Once an anaesthetist submits a List, office staff will review its Cards as a complete set, correct any issues in consultation with the anaesthetist, and authorise the List for billing.

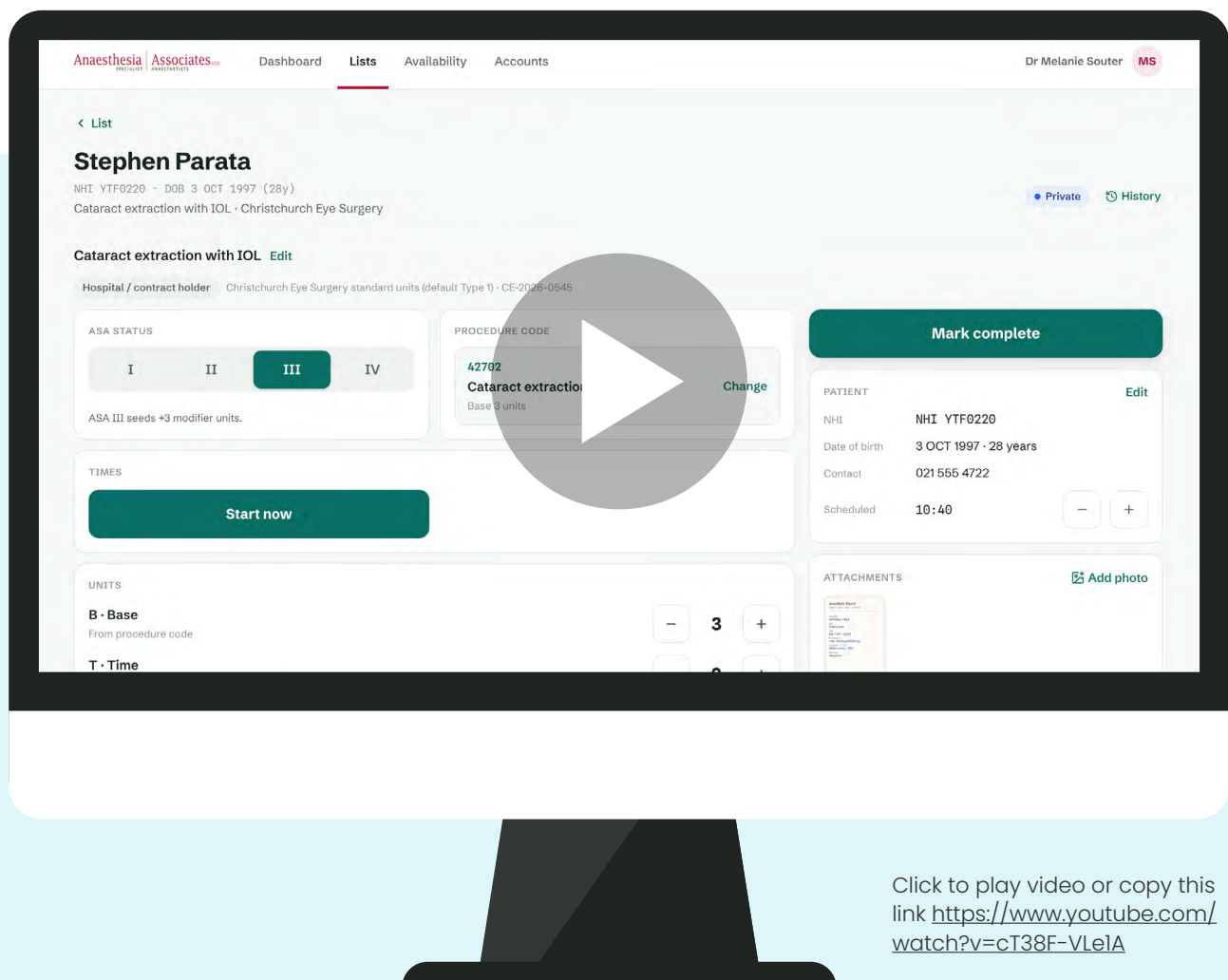
Authorisation passes the completed List to the Billing Engine, which applies AA's procedure, contract and billing rules, determines the appropriate payer, and generates the required invoices. Xero then operates behind the Billing Engine as the accounts-receivable, banking and payment service. Payments are collected into AA's account and the corresponding amounts due to anaesthetists are tracked separately through linked receivable and payable records. Role-based access, secure handling of personal information, complete auditability and clear exception monitoring will provide control throughout this end-to-end process.

App Prototype

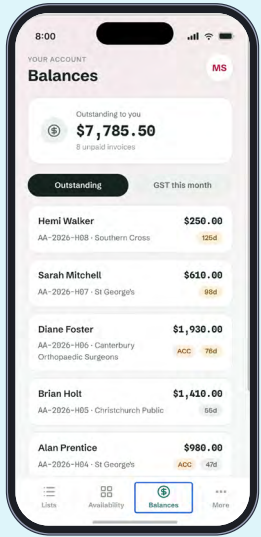
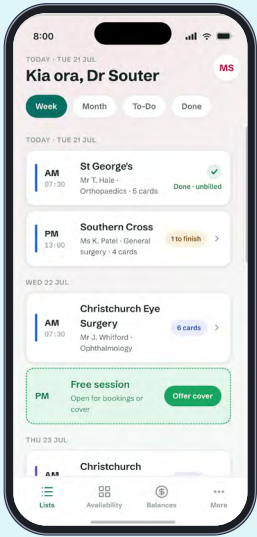
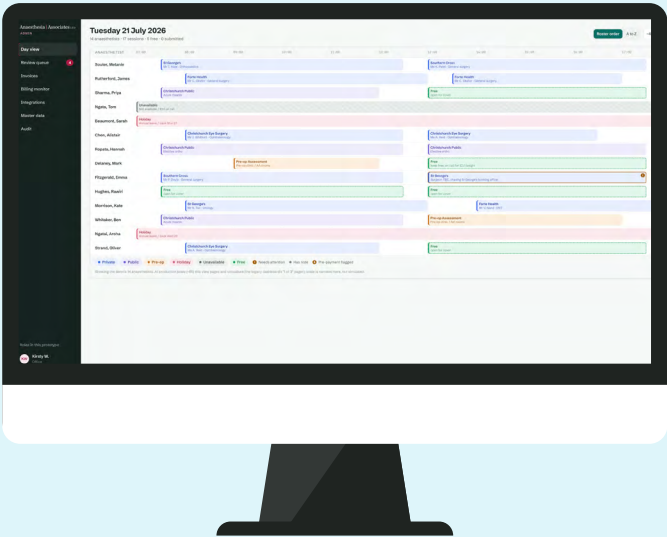
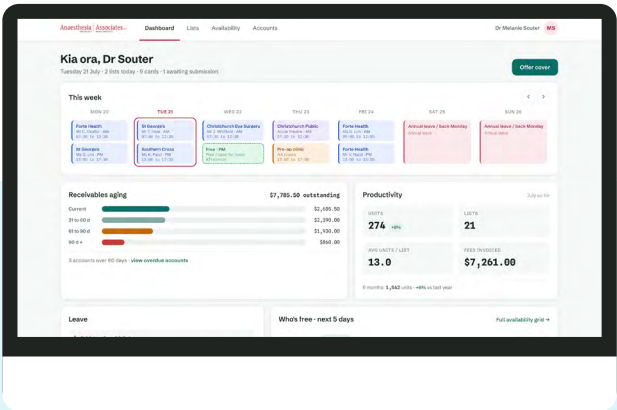
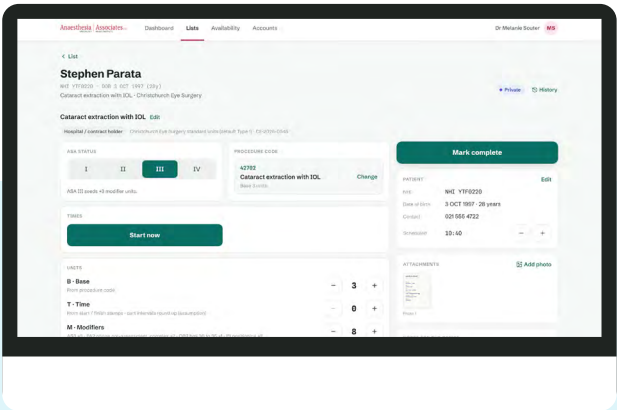
We believe that a picture is worth a thousand words. Wordy design documents can be difficult to understand and lead to mis-aligned expectations. As part of the Scoping and Design phase we will build an app prototype to demonstrate our understanding of requirements and give the client the opportunity to see how the system will actually work.

The user stories that make up the final requirements for sign off will be formulated from a combination of the prototype and discovery documentation.

Thanks to our investment in AI education and tooling we can quickly and easily build prototypes. As part of our RFP response we have built a sample application, based on the RFP requirements, to demonstrate this process.

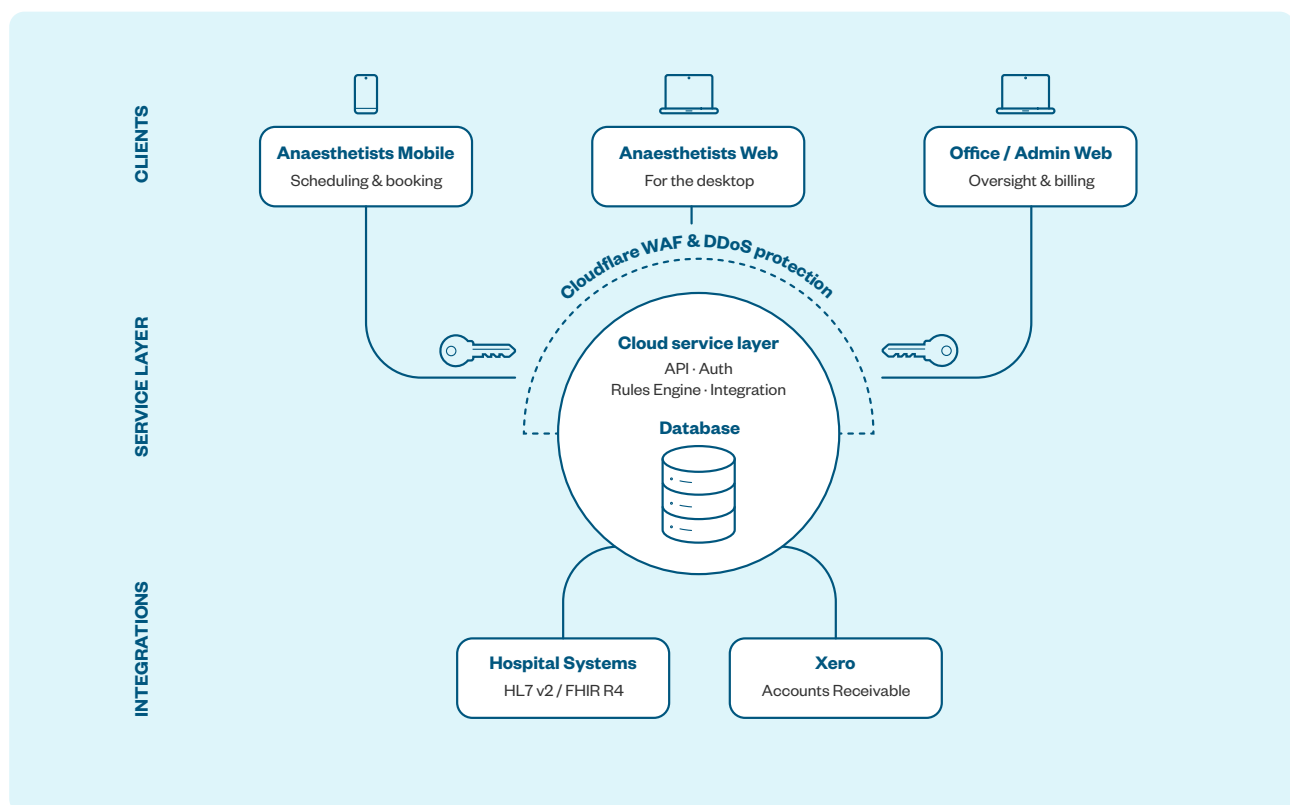


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High-Level Architecture

This section introduces the solution in plain terms: what it's made of, where it runs, and the key design principles. Our aim here is to give a complete picture without too much technical detail: the components involved; why we've chosen to host them the way we have; and how this addresses business risks. The more detailed sections that follow build on this foundation, going into the technical detail behind each of these approaches and addressing the specific open questions the RFP raises along the way.



The components

The solution will comprise:

Anaesthetist mobile application: The primary interface for anaesthetists: their calendar, their Lists and Cards, procedure detail, and the timesheet capture that feeds billing.

Anaesthetist web application: The same functionality as above, for desktop users.

Admin (office) web application: The interface for AA staff: handling manual changes from hospitals and surgeons' rooms, validating Card entries, and approving Lists for billing.

Cloud services, including the billing engine: The scheduling data store and the business logic that acts on it, with the billing engine as the system of record for what is owed.

Azure SQL database: The data platform beneath the cloud services, holding the schedule, billing, and audit history.

Integration layer: A dedicated tier connecting the system outward: to hospital systems for booking and clinical detail, to national health services for identity, and to Xero for accounts receivable and banking.

Cloudflare Firewall: Public traffic will be fronted by Cloudflare, providing geo-fencing to NZ-based traffic and automated blocking of unusual request patterns.

Xero: A dedicated instance providing accounts receivable, payment collection, and banking beneath the billing engine.

Hosting

The platform will be hosted in Microsoft Azure, a globally recognised and trusted cloud environment offering enterprise level security, compliance, and resilience controls. Azure provides physical, network, and operational safeguards including data encryption at rest, encryption in transit, and geographically redundant storage to protect against data loss and downtime.

Our intention is that all data will be hosted in Azure's New Zealand North region satisfying data residency requirements appropriate for NZ health information. As the New Zealand North data centre does not yet carry the full Azure feature set, we'll confirm its use during the discovery phase as we firm up the architecture.

All internet traffic into the system will be fronted by Cloudflare, a network security and performance platform, providing geo-fencing to restrict access to NZ-based traffic (if appropriate) and automated blocking of unusual request patterns – important protections for a system holding sensitive patient and business data.

Built for Change

The business model defined in the RFP is complex and rules heavy, especially around billing. We understand that while these rules are fixed today, they may need to evolve and change in the future. Data sources change and new standards emerge. We build software that is ready to change with the business – not being able to adapt to new demands or opportunities ignores one of the key value propositions of bespoke software.

In practice, this means a few things:

- **We model the business, not just the data.** Rather than building a system organised around database tables and data flows, we invest in understanding the business domain. In this case, how anaesthetist scheduling and billing actually works, what the meaningful concepts and events are, and how business rules apply. This approach, known as Domain-Driven Design, produces code that maps directly to the way a business thinks and talks about their work. The practical benefit is that when processes change, the system can change with them without unpicking unrelated parts.
- **We build in a safety net for change.** Automated testing means that when we make changes, whether fixing a bug, adding a feature, or responding to a regulatory update, we get rapid confirmation that nothing unexpected has broken. For a system handling health data, financial transactions, and surgical lists, this is not a nice-to-have. It's what makes confident, ongoing change possible.
- **We treat design as continuous, not a one-time event.** Good software design isn't something that happens once at the start of a project and then gets executed. Requirements become clearer as the system is built and used. We build in the discipline to refactor and improve the design as understanding deepens, so the system remains clean and fit for purpose rather than accumulating the kind of technical debt that eventually makes change prohibitively expensive.

At Stratos, this is a central consideration in how we approach every engagement. We design systems from the ground up to remain changeable and maintainable long into the future.

Our Investment in AI

The practice of software development is changing more rapidly than at any point in the last thirty years. We are committed to investing and innovating in this area. We have an AI Steering Committee that sets direction and risk appetite, and an AI Working Group translates that into practice. Between them they maintain our AI Usage Policy, the list of approved tools, our recommended models, and internal communities of practice for the tools in heaviest use. Our engineers work with frontier models from Anthropic and OpenAI, alongside the tooling that makes those models useful in a professional engineering context.

Client data does not enter AI tooling. No client data, for example patient identifiers, NHI numbers, clinical detail, billing records, or production extracts of any kind are provided to AI coding assistants. Where we need realistic data to build and demonstrate against, we generate synthetic data, as we have done for the prototype accompanying this proposal. For AA, this means the sensitivity of your data has no bearing on how we use these tools, because the two never meet.

What this investment produces for AA falls into three areas.

- **Interface design happens in-house.** Achieving a polished interface has historically required a separate design workstream – mockups produced in isolation, handed over to engineers, then revised through a slow loop. That loop has largely collapsed. Our engineers now produce and revise working interfaces directly and can iterate on them rapidly to validate assumptions and workflows. Practically, it means you will see and react to real screens during the scoping phase rather than wireframes.

- **Automated testing at a depth that was previously uneconomic.** Comprehensive test coverage has always been the solid engineering decision, but the ROI wasn't always justifiable. That trade-off has shifted. A system carrying dense automated tests is one that absorbs change safely and confidently. All systems face change, and this system is no exception: HL7 interface variation across hospitals and regulatory shifts such as the NHI format transition. Test coverage is what makes those changes manageable.
- **Working software early.** The prototype presented in this RFP was built using these tools and synthetic data. It exists so that you can validate the scheduling and Card workflow against something tangible before committing to a build, and so that the open questions raised in this proposal can be resolved against a working system rather than a document.

Finally, and importantly: the tools amplify our engineers, they do not substitute for them. Complex work spanning multiple hospital systems, integrations and billing rules still depends on experienced engineers who understand the domain and the relationships within it. Every change we make passes human code review and developer testing before it reaches QA. That is how we hold the line on the design principles set out in this proposal.

Robust and Reliable Integrations

Modern business systems don't run in a vacuum. Effective integration between disparate systems is vital to removing friction and inefficiency. When data flows reliably between platforms, without manual re-entry, reconciliation, or the errors that inevitably accompany both, the operational benefits are significant.

For AA's scheduling and billing system this means being able to reliably ingest up-to-date patient and booking data using established health-sector standards (HL7 v2 and FHIR) as well as exchanging financial data with Xero for accounts receivable.

Stratos has direct experience integrating data between health systems, and we understand the discipline this requires. Health data is sensitive, the systems involved are business-critical, and integrations must be built to handle failure gracefully, because cloud services are occasionally unavailable and data will sometimes arrive in unexpected states.

In practice this means a managed queue of messages – Azure Service Bus – that holds each exchange until it is confirmed processed, retries automatically when an upstream or downstream system is briefly unavailable, and applies a zero-tolerance policy to anything it still cannot process. You don't lose data that can't be processed – the message is set aside and raises an alert for a person or persons to act on appropriately. The result is an integration layer that fails safely and visibly, bringing a human into the loop when necessary, rather than quietly losing or corrupting a record.

Protecting the Integrity of the Data

The AA scheduling and billing system holds surgical schedules and financial records. This is important data and we want to guarantee that data is never quietly lost, overwritten, or altered after the fact. Three key design decisions, introduced here and detailed later in this response, work together to achieve that:

Safe concurrent change. Cards and Procedures can be edited from several places at once – an incoming message from the hospital booking system, the anaesthetist's mobile, or the office admin web application, right up to the day of the procedure. The system will use a standard "optimistic concurrency" model so that two people editing the same record cannot silently overwrite one another; where a genuine conflict arises, it will be surfaced clearly for resolution rather than settled by chance or ignored.

A complete change history. Every change to key scheduling and billing data – who changed what, and when – will be retained using Azure SQL's system-versioned (temporal) tables, giving a full history rather than a single "last modified" stamp.

Finalised invoices protected from change.

When a List is finalised for billing, the anaesthetist's rate, contract terms, party details, and any other appropriate details that applied at that time will be copied into the billing record, so that later changes to reference data cannot rewrite an invoice that has already been issued.

Security and Privacy

The system will be built to comply with HISO 10029 as a baseline requirement. All data – including patient and practitioner information – will be encrypted at rest and in transit. Role-based access controls will be strictly enforced at the service layer, ensuring users see only the data their role permits.

Application Security and Development Practices

- **Secure Development Lifecycle (SDL):** Security will be integrated throughout design, development, and testing phases, including code reviews and dependency scanning.
- **Penetration Testing:** Independent third-party security testing will be commissioned prior to go-live and whenever significant changes are made. In addition, we suggest that they be commissioned on an annual basis.
- **Data validation:** Strict input validation and sanitisation will prevent injection, cross-site scripting (XSS), and other common web vulnerabilities.
- **Audit and Logging:** Comprehensive logging will support system integrity checks and forensic analysis if required.

Detailed Solution Design

Scheduling, Billing, and the Data Model

We will adopt AA's candidate architecture – the schedule canvas grid, the List/Card hierarchy, half-day availability, and permanent-list templates – as a well thought out starting point. This section sets out only what we would add to it, and our answers to some of the open questions AA raised.

During discovery we'll spend time verifying the Schedule Management and Billing Engine candidate architectures proposed in the RFP. Here are the changes we currently expect to propose.

- **Reference data by identity, with one deliberate exception.** Hospitals, surgeons, statuses, rates and the like will be referenced into the schedule, not copied into it – changed once, reflected everywhere. We propose a single exception in the billing model, where reference data is frozen at authorisation so an issued invoice cannot be rewritten by later drift. We'll determine exactly which fields to do this for during discovery.
- **Automatic canvas generation on join.** When an anaesthetist joins, we will generate their forward canvas automatically, so coverage exists before any booking activity begins.
- **Snapshot at lock.** When a List reaches AUTHORISED, the engine copies every piece of reference data the calculation depended on – contract terms, rates, RVG values, the anaesthetist's unit value, party details – into the billing record itself. An invoice then stays connected to exactly what was true when it was raised, even after a contract is renegotiated or a rate revised later.

1 Open Question 1: a Card or Procedure being modified concurrently by multiple sources

The RFP quite rightly identified concurrent changes from different users or sources as a risk. A Card or Procedure is editable from several sources (surgeons' rooms, hospital integrations, anaesthetist's app) up to the day of the procedure. We use a standard approach to remove any risk of near-simultaneous edits silently overwriting one another, and a correction someone believed they made disappearing.

Our recommendation here is Optimistic Concurrency, not locking. Every source edits freely and conflicts are caught at save time.

The alternative to this is locking records before editing, but it's not recommended as it can be a lesser user experience and introduces the risk of records remaining locked accidentally.

Here's how it works:

- **Row-version backstop in Azure SQL.** Each Card and Procedure carries a row version. It's included when data is read from the database and checked against the read version when updates are pushed back to the database. This makes silent data loss impossible even for edits arriving at the same instant.
- **Resolved in the domain first.** The domain model decides whether two changes truly conflict. Non-conflicting edits (different fields) are merged and both kept; only genuine same-field contention is escalated to a person, as a clear, actionable message – so a raw database rejection never reaches a user as a technical error.
- **Auto-resolve vs escalate is a business policy (for discovery).** Which fields the system may auto-apply and which must stop for a person is a judgement we would map field-by-field with AA.

2 Open Question 2: mechanism for reassigning a List between Anaesthetists at short notice

Illness cover means a List and its Cards sometimes must move to another anaesthetist on or near the day. If this is done as a manual re-key of a new Card or Cards, the audit history of the Cards is lost. That destroys the record of who worked on what. Here's how we propose to address this:

- **Owner as a reference on the List aggregate.** The owning anaesthetist will be a reference held by the List – the root to which its Cards, statuses, attachments and history are anchored. Reassignment changes that owner reference to a new anaesthetist, so the whole List moves intact – including its full change history.
- **One audited event.** The move is captured as a single temporal-table event (from, to, by, when) that adds to history without rewriting it.
- **Two-slots-per-day invariant preserved.** The swap consumes the covering anaesthetist's existing half-day slot rather than adding a third List, and the vacated slot returns to available (or is marked unavailable if more appropriate) – so the calendar still renders without gaps or duplicates.

Remaining open questions - resolving these with AA

The two questions above are questions of mechanism: how software prevents concurrent writes from losing data, and how a List moves between anaesthetists without losing its change-history. Those we can answer from engineering first principles.

The remaining open questions in the RFP are questions about AA's business policy, and we cannot answer them honestly from outside the practice. The RFP itself flags several of them as matters for detailed design, and we agree. Below is what we understand each to hinge on, and where we currently sit.

Open question	What the decision turns on
Reconciling Anaesthetist Availability and Hospital Holiday data against the schedule canvas – hard constraints, soft warnings, or something else	Theatres open for emergency work and anaesthetists are asked to cover at short notice. Given the need to accept bookings up to the start of a session, a rule that cannot be overridden will simply be worked around outside the system. We lean toward interrupting the booking and requiring an authorised administrator to confirm with a captured reason. What we need from AA is how much friction each warrants, and where the line for “authorised” sits.
Whether the Insurer billing route needs its own rate table structure	Insurer terms tend to be schedule-based – a maximum payable per procedure – rather than a discount applied to a counterparty. So the real question is the shape of the rate lookup, not whether Insurer is special. We lean toward extending how the Contract model resolves rates rather than building a parallel structure alongside it. What we need from AA is how likely direct insurer billing is to grow, and whether cover split between an insurer and the patient is a real case for you.
Whether the Hospital billing route is final once resolved, or falls back to the Billable Party where a hospital disputes or fails to pay	This is a commercial and relationship decision before it is a technical one, and it has consequences beyond the invoice: re-routing an issued invoice affects the matching ACCPAY position and the anaesthetist's outstanding balance view. We would want to understand what AA does today before proposing what the system should do.

Patient and Billable Party deduplication and archiving at scale are already addressed by following the approach set out in Appendix 2 of the RFP.

Leaving these questions open does not leave them risky. Because we model the business domain rather than building around a fixed set of database tables, decisions like these become a rule added or adjusted within an existing concept, not as a restructuring of the system around it. A Contract that resolves rates differently, an override that carries a reason, a billing route that can fall back – each is a change within the model, proven by the automated test coverage that confirms nothing else was broken by the change. This is what we mean by building for change, and it is why resolving these questions properly during Scoping and Design, or in the future as our and AA's understanding grows, costs less than guessing at them now.

Audit and reproducibility

AA requires a full change history and the ability to reproduce any invoice long after the fact, across roughly 28,000 invoices a year. The RFP correctly identifies that a single “last modified by” stamp can't answer how a billing route changed, or what a Card looked like before a late reassignment, and it points toward “an append-only change log or event history” as the direction.

We propose to use Azure SQL temporal tables, at the data layer, to create that audit log. Because the database records history automatically, no write path can skip or bypass it, and point-in-time reconstruction (“what did this Card look like at that instant”) is a direct query rather than a log replay.

Alternatives considered. Trigger-based audit tables are workable but more bespoke and weaker at point-in-time reconstruction. A domain-event log captures the business why behind a change more richly but is less convenient as a queryable state record – it may still earn a place for cases where the reason for a change matters, but it isn't the primary history mechanism.

Integration and Interoperability

AA needs up-to-date booking data ingested reliably from hospital systems (HL7 v2 today, FHIR increasingly) and financial data exchanged with Xero. Reliability means resilience against transient network outages and visibility when messages are not delivered or do not contain the expected data.

Health systems integration - how we will build it

We'll build receipt and interpretation of electronic messages from hospital systems to be resilient by design.

Receipt and interpretation of messages from hospital systems is where the current pain sits, so we have designed this layer to be resilient by default.

Timeliness starts with transport. Today's messages land on an SFTP drive and are processed in batches, which is the source of the latency AA has identified. The batching, rather than SFTP itself, is likely what costs the time. We propose that the system poll the existing drop at short intervals and process each message the moment it lands, bringing latency down from hours to under a minute without requiring any hospital to change anything. Changing transport and protocols to an HTTPS endpoint for HL7 v2 sites, FHIR Subscriptions or scheduled resource polling for FHIR sites - depends on each hospital's integration team building an outbound interface on their own timetable. We would rather AA went live on near-real-time processing immediately and plan for hospitals to change progressively than have go-live wait on other organisations' resource availability.

Transport is an adapter, nothing more. Whichever way a message arrives, it enters the same queue and follows the same path from there. Moving a hospital from polling to push, changes one adapter, but leaves the queue, retry policy, failure handling and the translation (described below) all untouched. The per-hospital picture - v2 or FHIR, polled or pushed - becomes a configuration matrix we maintain, not a series of architectural decisions.

We use a durable message queue - Azure Service Bus Queues - between the outside world and the core of the system. This decouples receipt from processing. Once a message is accepted it can't be lost, even if a downstream component is briefly unavailable, restarting, or under load.

We will build exponential-backoff retries into the queue processing to absorb the large class of transient failures (a network blip, a service restarting), very often with no human involvement.

Malformed messages will fail all attempts at processing and eventually end up on the dead letter queue. We propose a zero-tolerance dead-letter queue. A message that still cannot be processed after retries is set aside with its failure context and raises a critical alert to office staff - every dead-lettered message represents a real booking change, so none is ever lost silently.

Silence is a failure too. Retries handle messages that fail; dead-lettering handles messages we cannot understand. Neither catches the message that never arrived. We propose to solve that with an expected-traffic profile per hospital feed and raise an alert when a site goes unexpectedly quiet. A cancellation that never arrives means an anaesthetist attends a procedure that is not running, and that failure is invisible unless something is watching for it.

We're proposing a slight change to the RFP suggestion in support of multiple formats (HL7 v2 and FHIR).

We propose all messages are translated to our own internal data model, not to a messaging format. We define the fields the system actually needs - the canonical booking, patient, and practitioner data - and write translators from each source format into that model. The scheduling core, validation, and audit never see HL7 or FHIR.

Validate on the data model, not the message.

Validation runs against the internal model, so it is one consistent gate however a change arrived - and we avoid a double-hop of translating to FHIR and then translating again. A messaging format is for transport; the data model is what the rules should be checked against.

NHI dual-format compliance

Health NZ's new NHI format carries a firm obligation: every system handling NHIs must support both the current and new structures, and have completed compliance testing, by 1 July 2027. The AA Booking and Billing System will meet that from go-live.

We already have experience with both formats and have existing systems that already support them in readiness for next year.

The NHI will be encapsulated in a single domain value object. It parses both structures, selects the correct check-digit algorithm from the identifier's shape and cannot be constructed in an invalid state, so an invalid NHI cannot enter the system at all. Because new identifiers are issued randomly rather than sequentially, the value object also removes any temptation for surrounding code to assume ordering or numeric ranging. It is the one place to change if the specification is refined; everything else depends on it and inherits its correctness.

Around it sits dual-format regression testing across every NHI-handling path: patient registration and record matching, NHI lookups via the Health NZ Digital Services Hub, inbound HL7 v2 and FHIR messages, and reporting.

Health NZ's published compliance test confirms a system can read and write the new format; we will complete it in our test environments ahead of go-live, and our own regression suite goes considerably further. Our domain object will be backed up by a strong suite of unit tests to prove the correctness now and into the future. Grounded in Health NZ's published specifications, this is a small, contained, well-understood feature to implement and prove.

Xero integration - how we will build it

Having accurate and auditable financial transactions is essential. Stratos has delivered Xero integrations across multiple production systems. We bring an understanding of both the technical and accounting mechanics that will ensure a reliable integration.

The billing engine is the system of record; a dedicated Xero instance acts as an accounts-receivable and banking add-on only, not AA's general ledger.

We build Xero integrations to be resilient by design:

- **Asynchronous processing:** Xero calls are handled outside the main application request cycle, so a slow or unavailable Xero API does not impact the user experience or block workflow progression unless necessary.
- **Durable message queues:** As with inbound messages, every financial event is written to a queue before it is sent to Xero. If a call fails, it retries automatically with exponential backoff. Nothing is lost.
- **Idempotency controls:** Each transaction carries a unique reference, so retries never result in duplicate invoices.
- **Sync status visibility:** AA staff users will have clear visibility of Xero sync status for each record, with failed items surfaced for review rather than silently dropped.

A noted inconsistency in the RFP, and the side we have taken.

The RFP is internally divided on the NHI: its NHI design-policy section proposes storing the NHI as a searchable cross-reference field on the Xero contact, whereas Appendix 2 states the NHI never leaves the PMS. We have followed Appendix 2 – the stricter reading – so nothing clinical crosses into Xero and the NHI's exposure is minimised to the fullest. The one cost is the convenience the other reading offered: looking a patient up in Xero directly by NHI. Under our model that lookup happens in the billing system instead, which we consider a fair trade for a smaller NHI footprint. We recommend the stricter posture but will confirm the choice with AA in discovery.

Billing integrity - where the two systems can diverge

The RFP sets the billing rules out clearly. What it says less about is what happens when an invoice is wrong, or when a run half-succeeds. With the billing engine as the system of record and Xero holding accounts receivable, the same financial event exists in two places, and there are situations where the two can drift apart. These are cases we would want to work through carefully with AA during Scoping and Design, since the right answer to each is as much an accounting and operational decision as a technical one. Some we anticipate:

- **An invoice needs correcting after authorisation.** Because an authorised invoice is never rewritten, correction means a credit note – and it has to move both sides. A credit against the ACCREC without a matching reversal of the ACCPAY leaves the anaesthetist's ledger position out of step with what AA has actually collected.
- **A billing run half-completes.** One authorised List can produce several invoice pairs across different counterparties. If the fourth fails, the three already in Xero cannot be unwound. A pair created only on the ACCPAY side is money owed to an anaesthetist with no receivable behind it.

- **Someone changes an invoice in Xero.** Xero is a real accounting package that AA staff can log into. An invoice voided or amended there to settle a query is invisible to the billing engine unless something is looking for it.
- **A payment is recorded against an invoice the engine has since credited.** The two systems can arrive at the same record from different directions, and the order events land in matters.

Our expectation is that these are handled by tracking billing state per invoice rather than per List, so partial runs are resumable and visible; by treating the ACCREC and ACCPAY as a unit that completes or backs out together; and by extending the daily reconciliation poll already proposed for payment detection to confirm that authorised invoices still match what the engine issued. We would confirm the specific policy for each case, and who resolves it, during Scoping and Design.

“The same financial event exists in two places, and there are situations where the two can drift apart.”

Mobile Application and Notifications

Mobile Application

The RFP calls for a mobile application that allows anaesthetists to manage their calendars and capture information about the various procedures performed. There are two credible technical approaches to delivering this – a native mobile application, or a Progressive Web App (PWA).

Our recommendation is a Progressive Web App. For a closed, well-known user base of approximately 85 anaesthetists, a PWA delivers effectively the same day-to-day experience as a native app at materially lower upfront and ongoing cost.

Native Mobile Application

A native app is downloaded and installed from the Apple App Store (on Apple iOS devices) or Google Play Store (on Android devices). It is built specifically for each platform – iOS and Android – and runs directly on the device rather than in a browser.

Native apps have significantly higher upfront and ongoing costs than web alternatives – Apple and Google each require their own build pipeline, app store compliance, and keeping pace with regular platform updates.

We have experience building native apps in the health space and use an approach that keeps that overhead as low as possible by sharing a single codebase across both platforms, but some additional cost is unavoidable.

The main advantages of a native application, that can make it worth the cost, are a seamless notification experience, easier to discover and install from the app store – a process all users are familiar with, and access to device services such as geo-location.

Progressive Web App (PWA)

A PWA is a web application that – once installed to a device's screen – looks, feels, and behaves similarly to a native mobile application. It runs in the browser engine rather than natively on the device. The intent is to close the gap between a web app and a native app without requiring an app store, and the considerable overheads that come with native app development.

While PWAs are generally more cost effective, the trade-offs need to be considered. With a small well known user base – of around 85 anaesthetists – discoverability isn't a factor for this system. There is also no requirement for on device services such as location.

We propose that during the scoping and design phase we more thoroughly evaluate the possibility of a PWA over a native application. We'll work through the cost implications with AA and engage directly with users to validate our thinking.

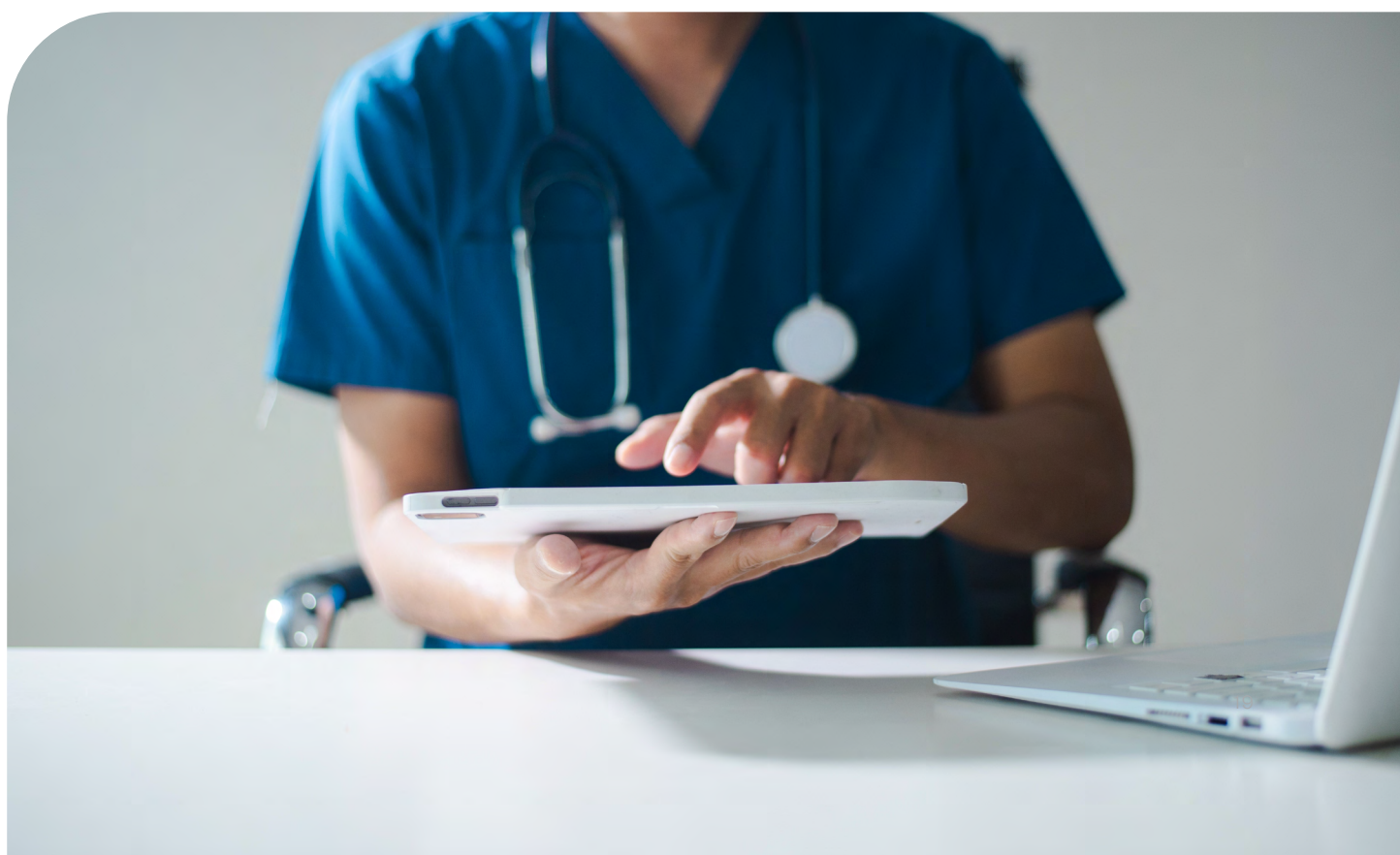
“We have experience building native apps in the health space.”

Authentication, Authorisation and Access Control

Secure authentication is critical, but so is making it easy for anaesthetists and administrators to actually log in. A cumbersome login experience is one of the fastest ways to undermine adoption.

We propose implementing a proven Identity-as-a-Service solution – either Auth0 or Microsoft's Entra External ID – for authentication and access management across the web and mobile applications. Both are cost-effective, battle-tested in health-adjacent environments, and invisible to the end user when done well. Importantly, both hold ISO 27001 and SOC 2 certifications, which are the internationally recognised security standards that underpin HISO 10029 compliance – so AA can be confident the identity layer of their platform is built on a robust, auditable foundation. We'll confirm the best fit during discovery.

- **Multi-Factor Authentication (MFA):**
We recommend enforcing MFA for Admin accounts given the sensitivity of financial and patient information held in the system.
- **Password management:** Anaesthetists can reset their own passwords securely and independently. Admins can provision accounts without ever having access to user credentials.
- **Every login attempt will be logged and auditable:**
Supporting AA's obligations under HISO 10029 and giving you a clean trail for any compliance or access review.
- **For anaesthetists on mobile, we'll additionally support social login via Google and Apple:**
These are secure, widely trusted platforms that most people already use daily – enabling a one-tap login experience that removes friction at exactly the moment it matters most.





Section Two

Security, Privacy & Compliance

Security Operations

A secure platform is fundamental to protecting patient and financial data and maintaining confidence across users. Our security operations model is designed to provide continuous protection, early detection of risks, and rapid response to emerging threats, while remaining cost-effective and simple for AA to govern.

Health Information Privacy Code 2020

The system holds health information, and that carries specific obligations under New Zealand law. Our design treats the Health Information Privacy Code 2020, issued under the Privacy Act 2020, as the governing instrument, with HISO 10029 as the security framework beneath it. The security operations described in the rest of this section are how we meet those obligations in practice.

The NHI is the most restricted element in the system, and is treated accordingly. It is held in the core platform only. It is used for the purpose it exists for with lookups performed against the NHI FHIR API via the Health NZ Digital Services Hub. It is not used as a billing key and does not cross into Xero, for the reasons set out in our Xero integration section.

We propose three design constraints to prevent health identifiers leaking through infrastructure rather than functionality:

- **Logs and telemetry.** The NHI is excluded from application logs, error messages, and monitoring telemetry. Diagnostic data references internal record identifiers, not patient identity.
- **Failed message handling.** A dead-lettered HL7 message contains a full PID segment, including patient identifiers. Where failures are surfaced to office staff for manual intervention, the working view presents only what is needed to resolve the problem, and the raw payload is held under restricted access rather than displayed by default.
- **Non-production environments.** Development, Test and UAT environments do not hold production patient data. They are populated with synthetic or de-identified data, so real NHIs exist in one environment only.

Support access to production is controlled and visible. Stratos personnel do not hold standing access to production data. Where investigation of an issue requires it, access is granted for a defined period and logged. All Stratos staff engaged on the platform are bound by confidentiality obligations, as required under the RFP.

Breach response is defined before it is needed.

The Privacy Act's notifiable breach regime requires the Privacy Commissioner and affected individuals to be notified where a breach is likely to cause serious harm. AA is the agency holding the information and makes the notification decision; Stratos is responsible for detection, containment, forensic analysis and providing AA with the factual basis for that decision, promptly enough to meet the statutory timeframe. We would agree the escalation path and contact points with AA during Scoping and Design.

Penetration Testing

Independent penetration testing is critical to ensure that the application is safe from any malicious actors who attempt to attack it.

- We suggest that an annual independent penetration test be carried out.
- Additional penetration tests will be scheduled for any major release or architectural change. Ideally these would fit in with the annual test schedule to remove duplication.
- Any issues raised will be assessed and appropriate action taken depending on the severity level.

“Proactive security operations designed for continuous protection and rapid response to emerging threats.”

Vulnerability Management

- Continuous automated scanning across code and cloud infrastructure using Azure Security Centre capabilities.
- Automated scanning of code libraries for known vulnerabilities using GitHub tooling.
- Stratos has a security committee who meet on a regular basis to assess security news and recent attack activity and how they might affect any of our client systems.
- Monthly patch cycle, plus fast-track remediation for any critical vulnerabilities that are identified.
- Azure SQL will automatically monitor the production database for unusual, or suspicious, patterns and activities.

Backups and Data Protection

Our data protection strategy includes:

- Automated daily full backups, with frequent incremental log backups.
- Encrypted backup storage (in transit and at rest).
- Quarterly recovery tests to validate availability and integrity.

In addition, we will configure Azure to automatically maintain geo-redundant backups, meaning:

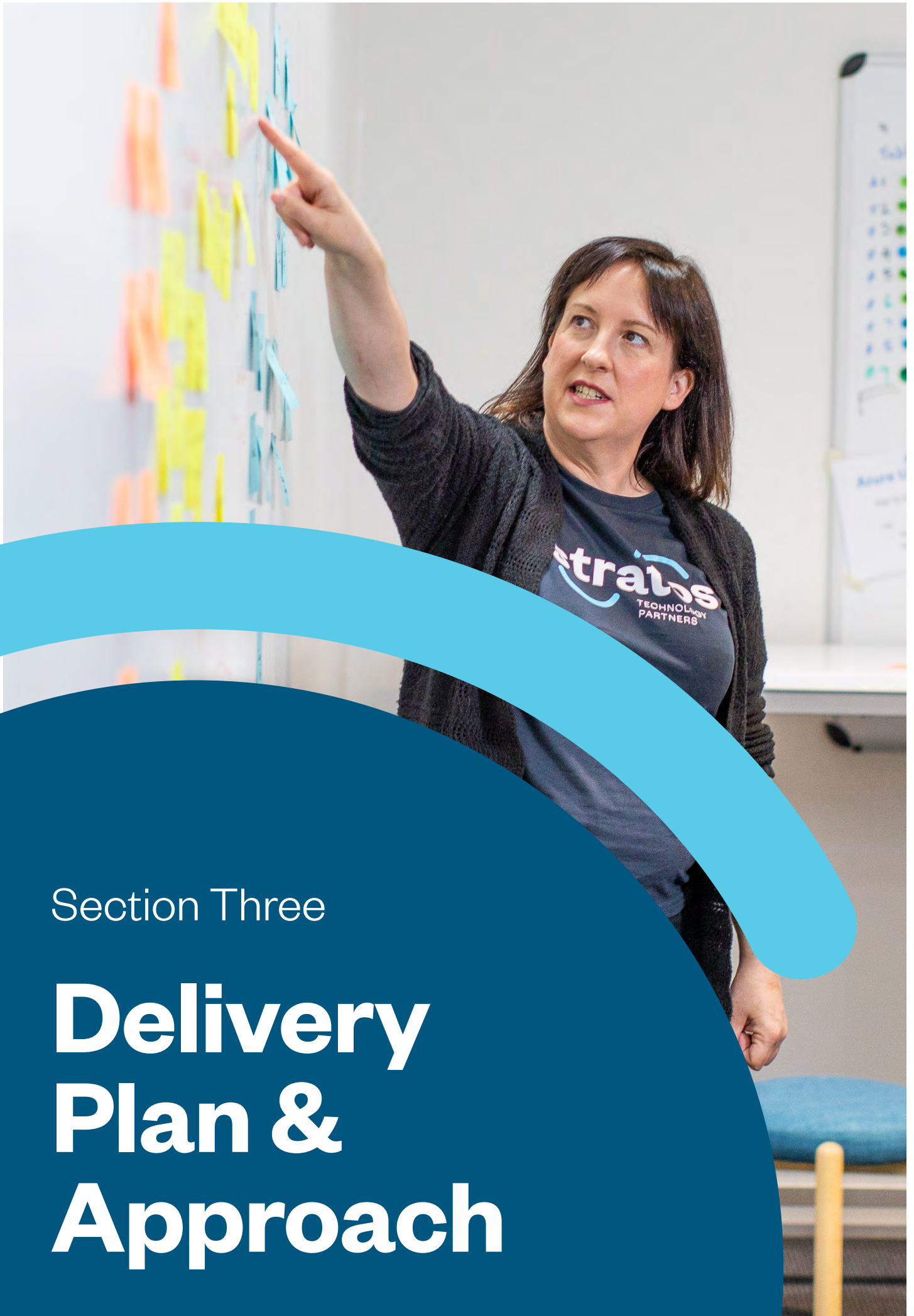
- Backups are copied to a secondary Azure region hundreds of kilometres away.
- In a regional outage, data can be restored from the alternate region.

Disaster Recovery

We will aim to be able to restore the database, should there be an event requiring this, within 2-4 hours from backups. The Azure database backup technology means that we can restore the database to a point in time (usually within minutes of a desired time) within the previous 35 days.

Long-term retention will be configured, allowing us to retain database backups for a maximum of 10 years.

We will work with AA to build a data retention policy that is cost effective and meets industry and system requirements.



Section Three

Delivery Plan & Approach

System Delivery Plan

Delivery Plan

Stratos follows a structured, yet flexible Software Development Lifecycle (SDLC) designed to ensure quality, predictability, and alignment with business outcomes. Our SDLC combines Agile delivery practices with formal quality and governance checkpoints to support transparency and consistent delivery.

The proposed delivery plan outlines a high-level milestone-based approach for delivering the AA Booking and Billing System, with combined Scoping and Design phase completed in 4 weeks, followed by 34 weeks of project implementation which will be organised into more detailed development and delivery milestones during the Scoping and Design phase.

We note the RFP proposes a two week Scoping and Design phase followed by a four to eight week implementation, and we have kept our approach as close to that shape as we responsibly can. We would not, however, be serving AA well if we agreed to a timeline we did not believe feasible. The overall scale of the build, and the complexity sitting behind the billing and integration requirements in particular, mean the full scope needs a longer implementation runway than the window set out in the RFP. The Scoping and Design phase will enable us to provide more accurate estimates.

Timeline Overview



Phase 1: Discovery and Scoping

Timeline: 17th August – 13th September 2026 (4 weeks)

The AA Booking and Billing System is a complex platform with multiple interdependent components and integrations. During the discovery and scoping phase, we'll work closely with AA to understand all the complexities of the business, answering the open questions from the RFP, and finalising the requirements.

We will iterate on the prototype with AA to build our understanding of requirements and improve the user experience of the final system. The user stories that make up the final requirements for sign off will be formulated from a combination of the prototype and discovery documentation.

We will confirm the technical architecture, firming up exact components as open questions are answered and requirements become more concrete.

Rollout is a critical consideration for a platform of this kind. Coordinating onboarding across anaesthetists, multiple hospitals, and the AA office team requires careful planning. During discovery and scoping, we will develop a detailed rollout plan, including the potential for a soft launch to validate the system in a controlled environment before full deployment.

Finally, we'll produce detailed costs for Phase 2 Project Implementation, alongside detailed cost estimates for ongoing operation and support that sit beyond it, giving AA a clear financial picture of the full journey, not just the next step, so future investment decisions can be made with confidence.

How decisions will be validated

We will run workshops with AA stakeholders to build a detailed understanding of each group's needs and workflows. This ensures the system we design reflects how people work, not just how processes are documented.

We'll translate that understanding into interactive mock-ups that test key workflows in real scenarios, letting dependencies, edge cases and operational constraints surface early. This ensures the chosen approach is based on observed behaviour and operational fit, not assumptions.

Anaesthesia Associates and Stakeholder Involvement

Participation from key stakeholders (practice leadership, office and booking administrators, billing and finance and representative anaesthetists) will be required for interviews, workshops and requirement validation sessions throughout the discovery phase. They will be required to review prototypes, validate workflows, clarify requirements and support decision-making.

Key Ceremonies

- **Project Kick-off:** Kick-off session with AA key stakeholders.
- **Stakeholder Interviews:** Stakeholder group interviews to understand needs and workflows.
- **Requirements Workshops:** Session with user representatives to understand detailed requirements.

“Designed around how people work, not how processes are written down.”

Phase 2: Project Implementation

Timeline: 14th September 2026 – 12th May 2027 (34 weeks)

The implementation phase focuses on delivering the core functionality required to run scheduling, billing and invoicing end to end from go-live, including web apps, mobile app, and integrations.

Milestone	Timeline
1. Development	14 th September – 14 th March 2027 (26 weeks)
2. Testing	15 th March – 11 th April 2027 (4 weeks)
3. Training and Change Management	15 th March – 11 th April 2027 (4 weeks)
4. Go-Live	12 th – 13 th April (2 days)
5. Hypercare and Parallel Run	14 th April – 12 th May (4 weeks)

Milestones and timeline to be confirmed during Phase 1.

Key Ceremonies During Project Implementation

- **Weekly Key Stakeholder Meetings:**
Status updates and decisions
- **Bi-weekly Planning: every 2 weeks:** Review upcoming work and priorities (Stratos)
- **Milestone Demos:** End of each milestone – demonstrate completed features to key stakeholders
- **Daily Stand-ups:** Development team coordination (Stratos)

Milestone 1: Development

14th September 2026 – 14th March 2027 (26 weeks)

- Foundation and user management
- Scheduling
- Billing engine
- Xero integration
- Hospital integrations
- Testing including automation, system, integration, and end-to-end, including mobile

Outcome: All core functionality is built and tested against the requirements agreed in Phase 1. Working software is demonstrated to AA at the end of development milestone, with a release candidate ready to enter formal security and user acceptance testing (UAT).

Milestone 2: Testing

15th March – 11th April 2027 (4 weeks)

- Performance testing and optimisation
- Penetration testing
- NHI dual-format compliance testing across all NHI-handling components
- UAT with key stakeholders and subject matter experts
- Bug fixes and remediation

Outcome: Testing is embedded throughout development, with this milestone providing the final, formal validation across performance, security, UAT and NHI compliance testing. It confirms the solution behaves correctly against the agreed functional and non-functional requirements, and concludes with UAT sign-off from AA stakeholders and subject matter experts validating the system is fit for purpose ahead of go-live.

Milestone 3: Training and Change Management

15th March – 11th April 2027 (4 weeks)

- Training sessions with user groups
- Support AA's change management ahead of go-live, with AA leading communications to its people

Outcome: User groups are trained in the new system, and the operational changes that come with it are planned and communicated ahead of go-live.



Milestone 4: Go-Live

12th – 13th April 2027 (2 days)

- Production deployment
- Smoke testing in production

Outcome: The system is live in production and AA is operating on the new platform, with trained users and the Stratos delivery team in immediate support through the hypercare and parallel run period that follows.

Milestone 5: Hypercare and Parallel Run

14th April – 12th May 2027 (4 weeks)

- Daily monitoring of system performance, error logs and user feedback
- Rapid response support during NZ business hours
- Priority bug fixes and performance tuning based on real-world usage patterns
- Parallel run of the legacy and new systems for one month, with AA recording bookings, billing and invoicing in both
- AA reconciliation of billing and invoicing outputs between the two systems, with any variance raised to the Stratos delivery team and investigated as a priority
- Parallel running of legacy system and new system to ensure confidence of accurate financials

Outcome: A stable system in production, closely monitored by the Stratos delivery team who are on hand to resolve issues quickly as users begin adopting the new system. This provides confidence the system is operating reliably prior to closing the project and moving into the support agreement. The parallel run gives AA independent confirmation that the billing and invoicing outputs of the new platform reconcile to the legacy system before the legacy system is retired.

Project Governance and Reporting

Our approach focuses on delivering valuable, working software quickly and consistently, while ensuring AA remains informed and in control throughout delivery. Progress is managed through two regular touchpoints:

- **Weekly Key Stakeholder Meetings:** Status updates and decisions.
- **Milestone Demos:** End of each milestone
 - demonstrate completed features to key stakeholders.

Alongside these, regular reporting gives AA visibility of progress against outcomes, budget utilisation, and any risks or issues as they arise. Day-to-day decisions are made by the delivery team so work keeps moving, and only changes affecting scope, budget or overall direction are escalated to AA for a decision.

Scope and Change Management

Scope is continuously refined and prioritised. Work only proceeds once clearly defined, understood and aligned to agreed priorities. Where new requirements or changes are identified, we assess their impact on scope, timeline and budget, discuss options and trade-offs with AA, and reach agreement before proceeding. If changes are expected to exceed agreed scope or budget, development is paused and re-aligned.

Quality Assurance and Testing

Quality is built into delivery from the beginning of development rather than left to the end. Each increment is defined with clear success criteria, then built, reviewed and tested as it progresses, and validated against the agreed workflows and business rules. Automated testing covers core business logic and critical workflows, ensuring later releases do not disturb existing functionality, and is supported by targeted manual end-to-end testing across real-world scenarios. End-to-end testing places particular emphasis on the hospital integration pathways and the Xero interface, with explicit coverage of failure and recovery scenarios – message delivery, error handling, retry and alerting – reflecting the reliability and operational visibility AA requires.

This is backed by a formal assurance stage once development is complete, covering security, performance, integration and end-to-end testing across the system as a whole.

A dedicated user acceptance testing period then gives AA stakeholders and subject matter experts the opportunity to validate the solution in a controlled environment and provide formal sign-off before release to production.

DevOps and Environments

Our DevOps approach ensures the new AA booking and billing system is stable, secure, predictable, and easy to operate. It will provide reliability, scalability, strong governance, and low ongoing operational overhead.

Environments

To ensure quality and avoid surprises, the system will progress through four controlled environments:

- Development, where our team builds and experiments safely without impacting real users.
- Test, where scheduling logic, billing and invoicing calculations, hospital and finance integrations, and system notifications are checked in detail, including failure and recovery behaviour.
- User Acceptance Testing (UAT), where AA stakeholders and subject matter experts validate real-world behaviour against their own workflows before sign-off.
- Production, the live environment used day to day by AA. This will be secure, actively monitored, and performance tuned for real world use.

How We Release New Features (CI/CD)

Enhancements will be delivered in small, predictable updates where possible, rather than large, high-risk releases. Each change will be:

1. Built and reviewed
2. Automatically checked for issues
3. Tested in a controlled environment
4. Reviewed and accepted by AA in UAT
5. Approved and released to Production

The build and deployment pipelines of the software will be fully automated and take place in a cloud environment so that it is reliable and reproducible.

Infrastructure-as-Code (IaC)

IaC means the entire system infrastructure: servers, databases, integration components and security settings, is defined in reusable, version-controlled templates rather than set up manually.

There are some very good reasons why this approach is important:

- **Reliability & Consistency:** Every environment is created the same way, removing human error. This guarantees consistent behaviour across Development, Test, UAT, and Production.
- **Faster Recovery:** If a problem occurs, the entire system can be rebuilt quickly and accurately using the same template. This dramatically reduces downtime risk and protects continuity of scheduling and billing operations.
- **Vendor Transferability:** Because the full infrastructure is documented in code, and the environment and software belong to AA, another provider could take the system over if ever required.
- **Full Auditability:** Every infrastructure change is tracked and reviewable, supporting the audit and traceability expectations that apply to patient and financial data.

Tooling

Area	Tools(s)	Why it matters
Cloud Hosting	Azure	Secure, reliable, scalable
Infrastructure	Terraform / Bicep	Consistency, rapid recovery, vendor transferability
Code & Collaboration	GitHub (owned by Microsoft)	Auditability and quality control
Deployment	GitHub Actions	Predictable, low-risk releases
Testing	xUnit, Postman	Confidence and correctness
Monitoring	Azure Monitor, App Insights	Real-time health visibility
Security	Azure Security Centre	Continuous checking of environments and traffic for security issues

Training and Change Management

We build systems that are intuitive to use, and we train people properly on them. We run training sessions directly with each user group ahead of go-live, shaped around how that group will actually use the system. Where possible we embed guidance within the application itself, so help appears at the point it is needed and standalone documentation can stay short and practical. Confidence across a mixed group of users is the measure of success here, so ease of use and in-context help are a particular focus.

Change management itself sits with AA, and we support it. We work with AA to identify what changes for each user group, help shape the timing and sequencing around go-live, and make sure the rollout plan agreed during discovery reflects how people actually work. AA leads the conversation with its own people; we make sure the detail behind it is accurate.

Go-Live Plan

Go-live is designed to be low-risk and well-supported, with scheduling and billing continuing uninterrupted through the transition and a positive first experience for users of the new system.

Cutover

We will move from the old system to the new one in a carefully managed sequence:

- We will work with AA to identify the data that needs to be migrated from the existing systems (for example users and roles, anaesthetists, hospitals and sites, contract and billing reference data, and open or unbilled work).
- Scripts will be written to automatically extract, transform, and load the data.
- All data will be checked and validated ahead of time.
- A small group of AA users and administrators validate:
 - schedule and booking data accuracy
 - contract, rate and billing reference data
 - billing calculation logic, including tiered time and split billing
 - a sample set of invoices calculated end to end and reconciled against expected values
- A small number of users will test the live environment before wider access is opened. There will need to be an adequate representation from the main user groups – office and booking administrators, billing and finance, and anaesthetists. This will include confirming that training has been completed and support material is in place.
- Any adjustments are made as required.
- The data extract will be run again if necessary (should any new data have been added to the old system in the interim).
- The legacy system remains available in read-write mode for the duration of the parallel run, so that AA can record transactions in both systems and compare financial outputs. It is placed into read-only mode at the close of the Hypercare and Parallel Run phase, so nothing is lost or overwritten.
- Access is then provided to the new platform.

A Clear Rollback Option

Although we do not expect it to be needed, a rollback plan is prepared and agreed before cutover and remains in force for the duration of the parallel run. If a major issue were to arise that could not be resolved quickly, AA and Stratos would jointly make the call to:

- Pause AA's use of the new system
- Continue in the legacy system, which is already live and in read-write mode
- Ensure no data is lost, with the legacy system holding a complete and current set of records
- Allow scheduling and billing to continue without interruption

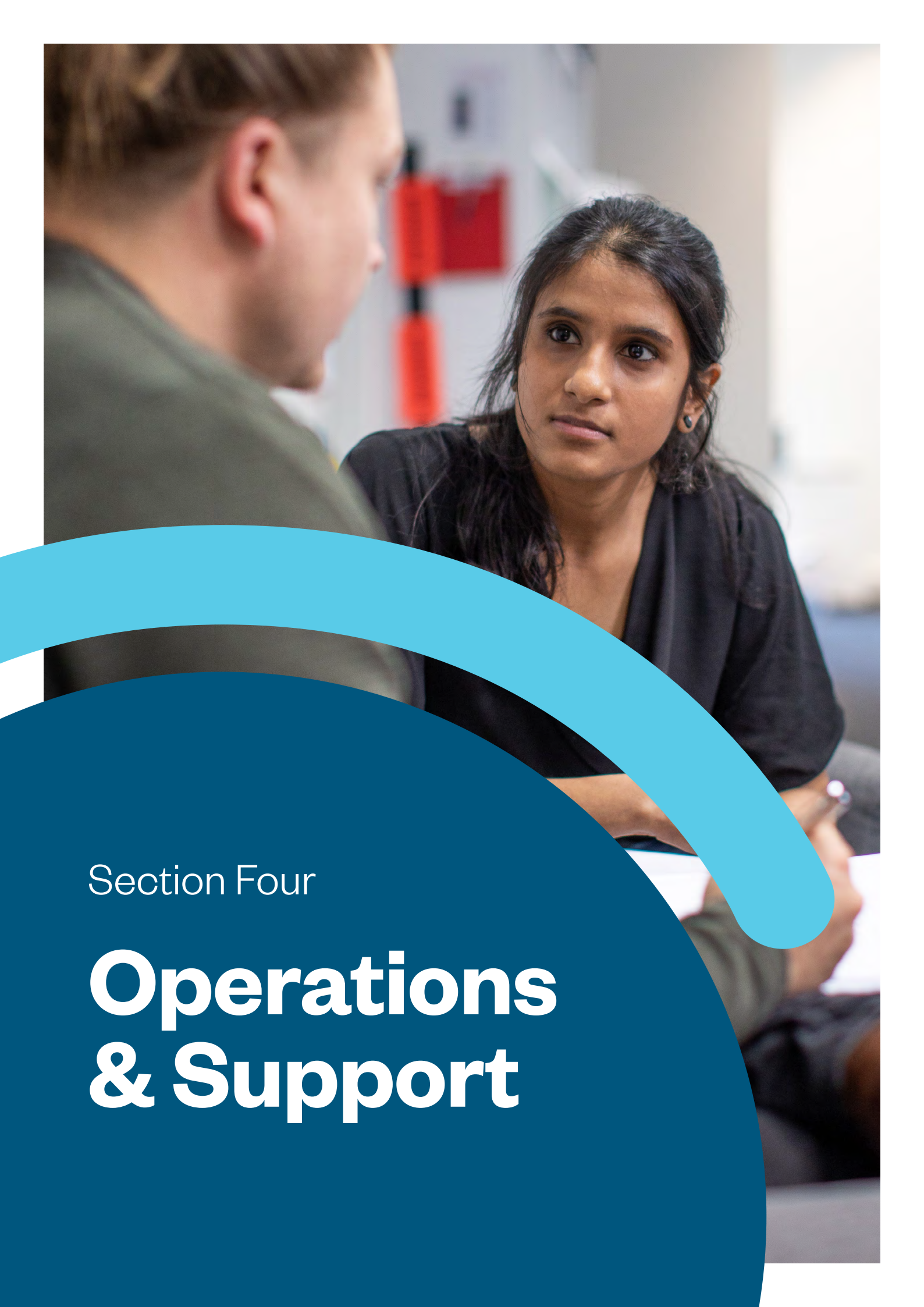
Because the legacy system remains live throughout the parallel run and is only placed into read-only mode at its conclusion rather than decommissioned, reverting is a low-risk step rather than a recovery exercise. AA would continue in a system its people are already using day to day, with no data migration back and no interruption to scheduling or billing.

Hypercare and Parallel Run: Stability and Confidence

For the four weeks following go-live we maintain a period of enhanced monitoring and support to ensure:

- System performance, error logs, integration activity and user feedback are monitored daily.
- Access, scheduling, billing and integration issues receive priority response ahead of standard service levels.
- Priority fixes and performance tuning are applied based on real-world usage patterns rather than waiting for a scheduled release.
- AA operates both the legacy system and the new platform side by side for one month, recording bookings, billing and invoicing in each.
- AA reconciles the financial outputs of the two systems including billing calculations, invoice totals, and revenue by anaesthetist, hospital and contract.
- Any variance is raised to the Stratos delivery team and investigated with Hypercare priority, then corrected or explained before the parallel run closes.

Hypercare and the parallel run end at the close of the four-week period, once the system is confirmed stable, the financials are confirmed to reconcile across both systems and AA is satisfied, at which point the legacy system is placed into read-only mode and the platform moves to the standard support model.



Section Four

Operations & Support

Support and Ongoing Partnership

Stewardship

Technical support is only one part of our ongoing partnership. As your business evolves, we'll help you navigate the technology lifecycle by providing practical advice on security, maintenance, enhancements and future investment. Rather than applying a standard approach, we work with you to understand your priorities and recommend the solutions that best balance risk, value and long-term sustainability.

Support and ongoing development will maintain system stability while continuously improving the platform post go-live.

Work is typically organised across a set of streams:

- **Support (reactive):** Issues, incidents, operational queries.
- **BAU maintenance backlog:** Stability, upgrades and non-functional improvements, as well as 'warrant of fitness' type system checks to surface new vulnerabilities and technology upgrade requirements.
- **BAU feature backlog:** Enhancements and new capabilities.
- **Projects:** Larger defined initiatives delivered via agreed Statements of Work.

These streams are prioritised and managed collaboratively with AA through regular operational sessions and a shared backlog.

AA will have clear visibility and input into:

- Active and upcoming work through a shared backlog and roadmap.
- Progress, priorities and activity across all work streams via structured reporting.

User Support

User support is typically managed through AA administrators as the first point of contact, with escalation to the Stratos delivery team where required. This keeps everyday questions close to AA's own process knowledge and keeps support costs predictable.

We can also offer direct end-user support as an option, where anaesthetists and office users raise tickets with the Stratos support desk themselves rather than routing everything through an AA administrator. This removes the triage load from AA, giving them a single tracked channel for access, booking and billing issues, and still gives AA administrators full visibility of every ticket raised. This can apply from go-live, or AA can start with the administrator-led model, then move to direct support later once usage patterns are clear.

Support Hours

Business-as-usual support:

- Monday-Friday, 8:30am-5:30pm NZT

Weekend or on-call support is not provided for this system by default, however given the requirement to support late bookings up to the start of an operating session, we can discuss coverage expectations during scoping.

Support Channels

All support requests are logged and tracked through a dedicated support portal providing:

- Visibility of ticket status and history
- Auditability for SLA tracking
- Input into future improvements and prioritisation

Tickets can be raised via the portal or through email/phone and are managed centrally by the Stratos support team.



Service Levels

The table below outlines our response timeframes where an SLA is in place. Support requests from clients who don't have an SLA are handled on a best-effort basis subject to team availability.

Priority	Description	Response (with SLA)
P1	System down with widespread business impact	1 hour response
P2	High-priority issue significantly affecting operations with no viable workaround	Same business day
P3	Standard issue or request with limited impact, or where a reasonable workaround is available	3 business days
P4	Low-priority issue, cosmetic defect, minor enhancement, or planned task with no operational urgency	Scheduled at a mutually agreed time

Incident Management

Incidents will be managed through a structured process:

- **Detection and Notification:** Issues identified through monitoring or user reports are triaged and escalated as required.
- **Containment and Restoration:** Priority is restoring service quickly, including temporary workarounds where needed.
- **Communication:** Clear updates at key stages of the incident.
- **Post-Incident Review:** Root-cause analysis for significant issues, with improvements fed back into future delivery.

A woman with long brown hair and black-rimmed glasses is smiling and looking towards the right. She is holding a black marker and appears to be writing on a whiteboard. The background is a blurred office or classroom setting with windows and lights. A large blue circular graphic is overlaid on the bottom left of the image.

Section Five

Commercials & Funding

Commercials

In this section we estimate the investment that will be required in the platform and potential commercial models.

We recommend structuring this engagement as a funded Scoping and Design phase under its own Statement of Work, followed by a separate Statement of Work for the build, with ongoing support and hosting covered by a further agreement.

Our preferred structure for the build is time-and-materials, which we can align to agreed delivery milestones once Scoping and Design has refined the scope, giving clear visibility and control over spend as work progresses.

The delivered cloud environment and software will belong entirely to AA and will be managed and operated by Stratos on their behalf. We will provide ongoing support to ensure the platform continues to perform reliably and securely for anaesthetists, facilities, and AA administrators.

We also note AA's interest in the potential for further sales of the product within New Zealand, and would welcome a conversation about a suitable commercial arrangement should AA wish to explore that; we make no assumptions in that regard at this stage.

All figures in this section are indicative, in New Zealand Dollars, and exclude GST.

Investment

Our experience in developing applications of this type, and particularly in the health sector, gives us confidence that we can provide a reasonable 'ball-park' estimate as to the investment that will be required for the development and ongoing maintenance of the system.

The range in our estimate reflects choices still open to AA. The largest variable is the mobile experience: a progressive web application sits at the lower end, while fully native iOS and Android apps deliver a richer offline and notification experience at significantly higher cost. Similar decisions apply to how many hospital integration formats are supported from go-live rather than introduced progressively once the platform is in production. We have presented a range rather than a single figure because we would rather work through these trade-offs with you than assume them. Our intention is to understand what fits within your budget and shape the delivery to match it.

We have included the initial development of the system and the costs that will be required for its ongoing support and development. We have not itemised each potential operating cost, but instead used real-world figures from very similar cloud-based systems that we run and host. We believe that this will give us the most reliable estimate.

Please note that these are very preliminary and will need to be revised after the Scoping and Design phase has been carried out.

Capital Costs

Item	Cost	Description
Scoping and Design phase	\$30k	Detailed business and technical design and analysis. Deliverables include a proposed timeline and milestones, firm capex and opex cost estimates, a development backlog to prepare for implementation, and concrete outcomes. Includes a polished prototype to support design and business analysis.
Project Implementation	\$300k - \$500k	Delivery of the designed systems (web app, services, mobile apps, per the proposed solution above) into a UAT environment; QA and end-to-end testing, with particular focus on integrations; support for AA through UAT; deployment to the production environment; 2 weeks post go-live hypercare support.
Penetration Testing	\$10-20k	Independent third-party testing prior to go-live.
Total Capex	\$340k-\$550k	

Operating Expenses

Item	Cost	Description
Hosting and Support	\$3-4k per month	–
Penetration Testing	\$10-20k p.a.	Independent third-party testing. Cadence and vendor to be agreed with AA.
Minor Upgrades / Improvements / Fixes	\$25-40k p.a.	Bespoke software requires ongoing maintenance and support, much like a car requires servicing. Without it, systems decay, becoming less secure and harder to change as new features are needed. We maintain systems on a regular basis – including security patching, dependency management, and planned upgrades – to keep them secure and supportable.
Total Opex	\$71-108k p.a.	

Licensing and User Management

As a bespoke, cloud-native solution, the platform is not licensed on a per-module or per-seat product basis, so there are no tiered product licence fees to manage. The costs that do apply are the underlying third-party and platform services, for example Microsoft Azure hosting, the dedicated Xero organisation, and SMS/email notification services, which are consumption-based and are already reflected in the Hosting and Support figure above. Any third-party licences required will be identified during Scoping and Design and passed through at cost.

Because there is no per-seat licence, adding or removing anaesthetists or office users does not incur a per-user licence charge. At AA's current and projected scale (approximately 85 anaesthetists plus office staff), user numbers have no material effect on the hosting cost above.

Professional Services Rates

We deliver on a time-and-materials basis at a single blended hourly rate covering the full project team, rather than separate rates per role. The rate below is indicative, excludes GST, and will be confirmed in the Statement of Work. The blend and number of resources engaged will be finalised following the Scoping and Design phase.

Resource	Indicative Hourly Rate (NZD, ex GST)
Blended project team (all roles)	\$185



Section Six

Project Team

Project Team Overview

Stratos has been delivering health solutions across Aotearoa New Zealand and Australia for the past 15 years.

This platform will be delivered by a dedicated, cross-functional team responsible for day-to-day delivery, governance, and communication with AA.

Behind this team sits the wider Stratos organisation including senior specialists, architects and support staff who can be drawn in at key moments to accelerate progress or unblock complexity, giving depth across software development, cloud infrastructure, AI integration and the Microsoft 365 and Power Platform stack. Stratos has delivered software into both private and public health, and these people have worked together as a team across a number of engagements, so they arrive with established working habits rather than forming from scratch.

“The right people at the table, with a bigger team behind them.”

Team strengths include

- **Full-stack development** across modern Microsoft and cloud technologies, including mobile applications.
- **Experience building rules-heavy operational systems** where calculation accuracy, auditability and data privacy matter.
- **End-to-end delivery capability and long-term support**, with a commitment to outcomes through iterative development.
- **A shared motivation** to do meaningful work for clients, clinicians, patients and the wider community.



Project Delivery Team



Donald Fraser
Team Lead, Product
Owner/Business
Analyst

Donald is a certified Business Analyst and Product Owner with over a decade of experience turning complex ideas into clear, user-focused solutions. Known for his approachable style and strong communication skills, Donald excels at connecting technical detail with business needs. He's passionate about understanding problems deeply and shaping practical, high-impact solutions that deliver real value. At Stratos, he helps guide product strategy and development, ensuring teams stay aligned and focused on what matters most to customers. Donald's curiosity and creativity are matched by his leadership and calm, solution-oriented mindset.



Campbell Cossens
Technical Lead,
Architect

Campbell is a full stack developer with over a decade of experience solving complex problems and creating software that makes a difference. He takes pride in writing clean, maintainable code and enjoys collaborating closely with clients and teammates to turn big ideas into reliable, long-term solutions. Equipped with a Bachelor of Science (BSc) from the University of Otago, Campbell focuses on both the technical details and the bigger picture. He's passionate about clear communication, bridging the gap between technical and non-technical audiences with ease.



Kris Little
Software
Developer

Kris is a passionate software developer with a love for building clean, intuitive, and efficient web applications. With more than six years' experience as a full-stack developer, he specialises in modern front-end frameworks such as React, Elm, and Angular. Kris began coding as a kid, creating small games that sparked his lifelong curiosity for how technology works. Now he has a Bachelor of Computer Science, and at Stratos he leads collaborations with clients and teammates, turning complex ideas into simple, elegant digital solutions. Known for his thoughtful approach and creative problem-solving, Kris brings enthusiasm and technical depth to every project he works on.



Peter Restall
Software
Developer

Peter is a seasoned software developer with more than 25 years of industry experience solving real-world problems through technology. Having spent two decades as an independent contractor in the UK, Peter brings a wealth of knowledge across multiple industries, tools, and frameworks. He enjoys tackling complex challenges, especially those with tangible outcomes that make life easier for users. At Stratos, he's passionate about delivering meaningful solutions for Kiwi businesses and mentoring others through his hands-on approach.

Project Delivery Team



Tom Lightfoot
Software
Developer

Tom is an enthusiastic software developer who brings curiosity, creativity, and a love for problem-solving to everything he does. As a recent graduate with a Bachelor of Computer Science, Tom has a strong foundation in modern programming practices, and thrives on learning new technologies and tackling challenges that stretch his skills. Since joining Stratos, he's focused on building robust and elegant solutions that make a real difference for clients. From the moment he wrote his first program, Tom was hooked, and that passion continues to fuel his drive to grow and innovate every day.



Josh Lloyd
Software
Developer

Josh is a software developer and tech lead at Stratos, focused on building systems that scale gracefully. With nearly two decades of experience, he combines technical architecture with practical problem-solving to create infrastructure that stands up to real-world demands.

Josh bridges technical and human challenges, collaborates across Stratos as a technical consultant, and has been exploring AI's potential within the company – emphasising thoughtful implementation over trend-chasing. (Microsoft Certified: Azure Developer Associate.)

Project Support Team



Greg Jackman
CTO and GM of
Software

With over 25 years in software engineering and 15+ years in software consultancy and services, Greg has worn a lot of hats – senior developer, business analyst, project manager, delivery lead, team lead, and CTO. Along the way he has worked across finance, healthcare, agriculture, and manufacturing, picking up well-earned experience that has made him pragmatic, calm under pressure, and laser-focused on getting working software into the hands of real users.

At Stratos, Greg works across the CTO and GM roles, guiding technical direction while making sure the custom software teams are set up to do their best work. Having sat in so many seats over the years, he has a feel for where projects get into trouble and what it takes to get back on track.



Bree Fick
Relationship
Manager

Bree is a Relationship Manager at Stratos who works side-by-side with key clients across our development teams. She engages early and often with executives, product leads and delivery teams to co-design practical plans that balance time, cost and outcomes. She keeps expectations aligned, delivery smooth, and steps in calmly when escalations appear. Her career has been spent working alongside operationally complex businesses on business and product strategy, with a focus on understanding how an organisation actually runs day to day and shaping roadmaps and investment decisions around that. She uses straightforward facilitation to surface needs and trade-offs and agree the next right step together.

A photograph of a surgeon in a blue surgical cap and mask, looking intently at a computer monitor in an operating room. The background is blurred, showing surgical lights and other equipment. A large, dark blue curved shape overlaps the bottom left of the image, containing the text.

Section Seven

Relevant Experience

The following case studies highlight Stratos' experience delivering modern healthcare platforms. The Gastro Booker project provides a close functional parallel to the platform Anaesthesia Associates is seeking, while the Sleep Health and Crohn's Colitis Care Australia platforms demonstrate our broader capability in clinical systems, patient-facing applications, and user-centred digital health solutions. All three platforms remain in active production and continue to be enhanced through our long-term partnerships with each organisation.

A Portfolio Of Specialist Clinical Applications Across Health New Zealand

For more than 15 years, Stratos has partnered with Health New Zealand to design, develop and support bespoke clinical applications that improve operational efficiency, streamline clinical workflows and integrate with existing healthcare systems. Our solutions are used across multiple specialties and districts, supporting clinicians, administrative teams and patients in the delivery of healthcare services.

NZ Health Portfolio of Systems

System	Clinical Area	Overview
Gastro Booker	Gastroenterology	Outpatient scheduling and patient flow management supporting referrals, clinic templates, bookings, reservations and operational workflows across specialist gastroenterology services.
Sleep Health	Sleep Medicine	Supports the end-to-end management of sleep assessments, patient workflows and multidisciplinary care coordination for specialist sleep services.
Respiratory Reporting	Respiratory Medicine	Enables structured clinical reporting and standardised capture of respiratory diagnostic information to improve consistency and accessibility of clinical data.
Apollo	Urology	Supports the long-term management and surveillance of urology patients through specialist clinical workflows and patient tracking.
IBDSmart	Gastroenterology	Remote patient monitoring platform enabling ongoing assessment and management of patients with inflammatory bowel disease between clinical appointments.
Pathology Billing	Pathology	Manages pathology billing and financial workflows, integrating operational and financial processes within the wider healthcare environment.

Healthcare Integration Experience

- HL7 messaging
- REST APIs
- Patient demographics via PMS
- Patient letter push to EMR
- Health One identity

Impact

Following more than a decade of collaboration with New Zealand's District Health Boards and now Health New Zealand, Stratos continues to work alongside the organisation as the management of clinical applications transitions to a nationally coordinated operating model. This work requires collaboration across product owners, procurement specialists, clinical teams and system administrators to establish governance and delivery practices for portfolios of related clinical applications. Our continued involvement demonstrates Stratos' ability to work effectively within complex enterprise environments and develop long-term collaborative relationships that extend beyond the delivery of individual projects.

The Gastro Booker Application

Health New Zealand's Gastroenterology Services required a solution to manage the complexity of outpatient clinic scheduling, referrals and patient flow across specialist services. Stratos worked with clinical and administrative stakeholders to develop Gastro Booker, a bespoke application that has supported gastroenterology services in Canterbury since 2015 and was later adopted by Nelson Marlborough. The platform is now entering its next phase, with work underway to roll it out to Southern Health New Zealand.

The application supports the day-to-day operation of gastroenterology services, including booking workflows, clinic management and configurable clinic templates. It provides functionality for scheduling calendars, rooms, sessions, procedures, booking slots and reservations, enabling services to manage both recurring clinic structures and day-to-day operational changes.

Booking staff, clinicians and administrators each have role-specific functionality within the same application, allowing them to manage their responsibilities through a single system.

Gastro Booker also forms part of the wider Health New Zealand technology environment. It integrates with hospital systems using HL7 messaging and REST APIs, with SQL Server providing the underlying data platform. Admissions are synchronised automatically through asynchronous processing, allowing clinical information to flow between systems without manual intervention.

More than ten years after its original delivery, Gastro Booker remains in production and continues to support specialist gastroenterology services. We have included this project because it demonstrates Stratos' experience delivering enterprise-grade scheduling platforms that support complex operational environments and integrate with existing enterprise systems.

A Smarter Clinical Pathway for Sleep Assessment and Decision-Making

The Challenge

The Canterbury Sleep Health Service supports patients across New Zealand with a wide range of sleep conditions, with a primary focus on obstructive sleep apnoea and sleep-disordered breathing. Their goal is to reduce patient risk and improve overall health outcomes through timely assessment, effective treatment, and strong clinical decision-making.

Sleep issues have far-reaching consequences. Beyond fatigue, untreated sleep apnoea is linked to increased risk of car accidents, workplace incidents, and reduced productivity. Demand for sleep services continues to grow, with the team seeing over 2,000 new patients annually—well above their ideal capacity.

To manage this demand, the service relies on a structured pathway involving community-based assessments, diagnostic sleep studies, and weekly multidisciplinary team (MDT) meetings. These MDT sessions bring together clinicians from multiple disciplines to review 50–70 cases in a single sitting and determine the best course of action for each patient.

While Stratos had already digitised the intake process, the MDT workflow remained heavily reliant on paper. For each patient, clinicians printed assessment forms and sleep study results, collated them into packs, and manually wrote notes during discussions. This process was reliable but time-consuming—taking several minutes per patient and consuming valuable clinical time on low-value administrative tasks.

Our Solution

Stratos partnered with the Canterbury Sleep Health Service to evolve their existing system into a fully paperless workflow, supporting both the assessment pathway and the MDT decision-making process.

Building on an earlier version of the Sleep App, the next phase focused on enabling clinicians to review, discuss, and document cases entirely within the system.

At the centre of the solution is a real-time, multi-user platform that supports the MDT meeting itself. Each clinician can access the same patient list on their own device, with the system coordinating activity across the group. As cases are selected, reviewed, and updated, that state is reflected live for all participants, ensuring everyone is aligned without relying on paper packs or manual coordination.

The updated platform now supports:

- End-to-end digital assessment and submission of patient data
- Real-time access to referrals, sleep studies, and structured questionnaires during MDT meetings
- Live coordination of patient lists across multiple clinicians and devices
- Direct entry of clinician notes into the system during discussions
- Improved navigation and usability to support rapid case review
- Automation of follow-up letter workflows, including bulk actions after MDT meetings

Previously, clinicians would leave the MDT with handwritten notes and manually process follow-up letters for each patient individually. The system now enables many of these letters to be generated and managed in bulk, significantly reducing post-meeting administrative effort.

Impact

- **Significant time savings:** MDT preparation reduced from several minutes per patient to under a minute, saving multiple hours of clinician time each week.
- **Reduced administrative burden:** Follow-up letter processes streamlined through automation and bulk handling.
- **Improved visibility and accuracy:** Notes are now typed, legible, and accessible within the system.
- **Better coordination in MDT meetings:** Real-time system ensures all clinicians are aligned during case discussions.
- **Increased reliability:** A shared digital record reduces reliance on paper and manual processes.
- **More time for high-value work:** Clinicians can focus on patient care and decision-making rather than admin.

The result is a more efficient and transparent system that supports clinical decision-making at scale, without compromising quality.

Looking Forward

Looking ahead, the focus is on continuing to reduce manual handling and improve the flow of information across the service. This includes enabling digital delivery of patient results and letters, strengthening integration with GP referral systems, and introducing smarter alerts to ensure no patients fall through gaps. There is also an opportunity to further streamline how data moves between systems and explore deeper integration of sleep study data. Together, these improvements aim to create a more connected, efficient pathway that supports clinicians to deliver timely care at scale.

Empowering patients to take an active role in their care

The Challenge

Crohn's Colitis Cure Australia (CCCure) supports a network of inflammatory bowel disease (IBD) centres across Australia and New Zealand, helping clinicians manage patients with Crohn's disease and ulcerative colitis. Their mission is to ultimately find a cure; in the interim, they are focused on improving how people living with IBD manage their condition day to day and enhancing overall quality of life.

For many years, CCCure has operated a well-established clinician platform, originally developed by Stratos as part of a long-term partnership that began in 2015. The platform is used at 16 locations across New Zealand and Australia, and captures detailed patient information across the full lifecycle of care, supporting clinicians in managing complex cases of Crohn's disease and ulcerative colitis.

The system enables structured recording of consultations, treatment plans, lab results, and clinical history, and integrates with external systems to bring together relevant patient data in one place. Over time, it has also supported research and population-level insights through the ability to aggregate and analyse anonymised data across sites.

Building on this strong foundation, developed a longer-term vision to create a more connected model of care, where patients and clinicians could work together using shared information.

The goal of the new application is to enable patients to engage more actively with their own health information, track their condition over time, and contribute meaningful data alongside clinical care.

The Consumer App Solution

Stratos worked with CCCure to design and deliver a new patient-facing application that extends the existing platform and introduces a more collaborative approach to care.

The initial focus was on replacing the existing patient portal with a mobile-first experience that is accessible, intuitive, and useful beyond a single clinical interaction. The application allows patients to complete pre-clinic questionnaires directly from their device, while also introducing a set of validated self-assessment tools.

These tools enable patients to measure key aspects of their condition, including symptom severity and quality of life, using clinically recognised scoring systems. Rather than simply submitting this information, patients can now track their results over time and build a clearer picture of how their condition is evolving.

The application also introduces the ability for patients to log day-to-day factors such as sleep, stress, diet, and activity. This creates a richer dataset that can be viewed alongside clinical assessments, helping to surface patterns and provide context for changes in symptoms.

This approach creates a two-way flow of information. Patients are better equipped to understand and manage their condition, while clinicians receive structured, meaningful data that can inform care decisions.

How We Work with CCCure

Our work with CCCure is built on a long-term partnership and a highly collaborative delivery model.

We engage closely with clinical leads and stakeholders to define outcomes at a high level, then work iteratively to shape and deliver features in small, focused increments. Requirements are refined close to implementation, ensuring that what is built reflects the latest understanding of user needs and clinical priorities.

Concepts are explored and validated early using mock-ups and AI prototypes, allowing stakeholders to visualise how the application will work in practice before development begins. This exercise clarifies requirements quickly, and ensures alignment across both technical and clinical perspectives.

The result is a delivery approach that balances a clear strategic direction with the flexibility to adapt as new insights emerge.

Impact

The introduction of the consumer application has changed how patients interact with the CCCure platform.

Patients can now engage with their health information in a more meaningful way, completing assessments on their own terms and tracking changes over time. This provides a clearer understanding of their condition and creates a stronger foundation for conversations with clinicians.

By using clinically validated tools, the data generated through the application is directly relevant to healthcare providers. Patients are able to present structured insights, rather than anecdotal information, which helps ensure their concerns are understood and taken seriously during appointments.

The platform also reduces reliance on one-off interactions, shifting towards a more continuous model of care where both patients and clinicians have access to a shared view of health over time.

Looking Forward

The consumer application is designed as an extension of the broader CCCure platform and will continue to evolve alongside it.

Upcoming activities focus on improving visibility and insight for patients, with the introduction of timeline-based views that allow individuals to see how their symptoms and assessments change over time. This will be supported by the ability to explore trends and identify patterns across different aspects of their health.

We will also introduce intelligent insights based on the data captured within the application, helping patients better understand potential drivers of their condition and supporting more informed discussions with clinicians.

Over time, the platform will continue to strengthen the connection between patients and clinicians, supporting a more collaborative, data-driven approach to care.

Stratos will continue to work alongside CCCure to ensure the platform evolves in line with clinical needs, patient expectations, and the broader goal of improving outcomes for people living with IBD.

Client References

As requested, two client reference sites are provided below.

CROHN'S COLITIS CURE

Name

Professor Jane M Andrews

Title

Board Chair and Medical Director of Crohn's Colitis Cure

Email

jane@c-c-cure.org

Phone

+61 417 814 828

Engagement summary

Jane and the team at Crohn's Colitis Cure have worked with us over a number of years, through that time Jane has been one of our main points of contact, engaging with our team on a weekly basis

System summary

See CCCure Case Study above

**Name**

John Foley

Title

Senior Production Agronomist

Email

jfoley@pggwrightsonseeds.co.nz

Phone

+64 27 220 3657
or direct +64 3 964 7305

Engagement summary

The New Zealand Seeds Authority and SCID Working Group began working with Stratos in late 2025 after Stratos successfully won a competitive RFP process. Since then, our teams have worked closely through weekly meetings, workshops and in-person sessions to understand the industry, define the scope and deliver a modern solution for Canterbury's arable sector. The project was delivered ahead of schedule and under budget, with a strong focus on accuracy, usability and an exceptionally polished final product.

System summary

SCID, or Seed Crop Isolation Distance, is an industry-supported GIS platform used by New Zealand's vegetable seed sector to map and manage seed crop plantings. It helps maintain genetic purity by checking the required isolation distances between crops, identifying potential cross-pollination conflicts and notifying affected parties so they can resolve them in accordance with the industry Code of Practice.



Appendices

Appendix A - Legal and Financial Basics

Appendix B - Certificate of Insurance

Appendix C - Stratos Policies

Appendix D – Certifications



Appendix A

Legal and Financial Basics

Stratos Technology Partners – Legal & Financial Basics

Company Name

Stratos Technology Partners Limited

Company Number

3344250

NZBN

9429031150002

Registered Office

Level 1
518 Colombo Street,
Christchurch, 8011,
New Zealand

Year Established

2011

Legal Status

Privately held New Zealand limited liability company

Directors

David William Carter
Steven Michael Knutson
Philip Neil Johnson

Core Business Activities

- Software design, development, and integration
- Cloud-native SaaS platforms
- Data and GIS solutions for regulated and primary industries
- Long-term system maintenance and support services
- Microsoft 365, SharePoint, and Power Platform design, development, and integration
- IT Managed Services
- AI consulting and development

Financial Overview (Indicative)

- Annual Revenue: NZD \$10.2M (31 March 2026)
- Staff: 60 (developers, project managers, data specialists, and support)
- Profitability: Positive EBITDA and stable revenue growth over the past three years
- No outstanding tax, compliance, or legal obligations

Financial Stability

Stratos has maintained profitability and consistent revenue growth since inception.

The company holds no external debt and maintains healthy working capital reserves to ensure project continuity and support for long-term contracts.

Insurance Cover

- Public Liability: NZD\$ 2 million
- Professional Indemnity: NZD\$ 2 million
- Cybersecurity and Data Breach: NZD\$ 2 million
- Employers' Liability and Statutory Liability: NZD\$ 2 million

Contracting Capability

Stratos regularly contracts with government agencies, large corporates, and industry groups, including multi-year service agreements for SaaS and support. Standard contractual frameworks include:

- Master Services Agreement (MSA)
- Software Development & Support Agreement
- Data Security and Privacy Addenda



Appendix B

Certificate of Insurance



CERTIFICATE OF INSURANCE

LLOYD'S CERTIFICATE OF INSURANCE

effected through

Delta Insurance New Zealand Limited

Level 22, 48 Shortland Street, Auckland, 1010. PO Box 106 276, Auckland 1143.

E-mail: underwriting@deltainsurance.co.nz

(hereinafter called the Coverholder)

THIS IS TO CERTIFY that in accordance with the authorisation granted under Contract UMR B60822025DELPL01, B6083172500386, B6091303221500025, B1216CB2600004, Dacred to the undersigned by certain Underwriters at Lloyd's, whose definitive numbers and the proportions underwritten by them, which will be supplied on application, can be ascertained by reference to the said Contract, and in consideration of the premium agreed, the said Underwriters are hereby bound, severally and not jointly, each for his own part and not one for another, their Executors and Administrators, to insure in accordance with the terms and conditions contained herein or endorsed hereon.

NAMED INSURED: Stratos Technology Partners Ltd

POLICY NUMBER / WORDING:	D17385/2026/ODO/1	Delta Optima 10-17 R0423
	D17385/2026/OEP/1	Delta Optima 10-17 R0423
	D17385/2026/OEL/1	Delta Optima 10-17 R0423
	D17385/2026/OSL/1	Delta Optima 10-17 R0423
	D17385/2026/OCR/1	Delta Optima 10-17 R0423
	D17385/2026/LEI/1	Delta Legal Expenses 10/19 R0923
	D17385/2026/TL/1	Tech 03-15 R0423
	D17385/2026/DOT/1	Dacred
	D17385/2026/ID/1	See Endorsement

POLICY PERIOD: Inception Date: 13 June 2026
Expiration Date: 13 June 2027 (4:00 pm NZ Standard Time)

LIMIT OF LIABILITY:

Directors & Officers Liability	\$500,000 Any One Claim and in the Aggregate
Employment Practices Liability	\$100,000 Any One Claim and in the Aggregate
Employers Liability	\$1,000,000 Any One Claim and in the Aggregate
Statutory Liability	\$1,000,000 Any One Claim and in the Aggregate
Crime	\$100,000 Any one single loss and in the aggregate
Commercial Legal Expenses	\$100,000 Any One Claim and in the Aggregate
Technology Liability	Errors & Omissions: \$2,000,000 E&O: Any one Claim, GL: Any one Occurrence (Products in the Aggregate) Personal Injury & Property Damage: \$2,000,000
Dacred Online Training Tool	\$0
Optima Infectious Diseases Extension	\$100,000 Any single Covered Event and in the Aggregate

UNDERWRITERS: Delta Insurance New Zealand Ltd for and on behalf of
Certain Underwriters at Lloyd's led by Allied World Managing Agency Ltd, Syndicate 2232
Certain Underwriters at Lloyd's led by SCOR Managing Agency Ltd, Syndicate 2015
Certain Underwriters at Lloyd's led by Antares Managing Agency Ltd, Syndicate 1274
Certain Underwriters at Lloyd's led by Tokio Marine Kiln Syndicates Ltd, Syndicate 0510
Dacred



COVERHOLDER NOTICE TO INSURED:

Agent of the Underwriters

In effecting this Policy, any cover is accepted by DELTA INSURANCE NEW ZEALAND LIMITED (DELTA) acting under an authority given by the subscribing Underwriters and DELTA is acting as Agent of the Underwriters and not the Agent of the Insured.

IN WITNESS WHEREOF, the Underwriters have caused this Policy to be signed on the Schedule by a duly authorised representative of the Underwriters.



Delta Insurance New Zealand Limited

Authorised Representative

Policy schedule issued on 15 June 2026



Appendix C

Stratos Policies

Stratos Policies

Stratos AI Usage Policy

Intent

The intent of this policy is to ensure clear understanding of Stratos' stance on the usage of Generative AI tools to aid in work related tasks.

Working with AI tools

At Stratos, we support you using AI tools to help you work better. However, when using AI tools there are a couple of things that you need to bear in mind:

- Consider security and privacy of both company and client data.
- Client AI policies, when agreed, override ours.

Before using an AI tool, you must consult the current list of approved AI tools at <https://stpnz.atlassian.net/wiki/spaces/Stratos/pages/131432449/Stratos+Approved+AI+Software> and use them in accordance with the usage tier that they have been assigned to ensure proper data governance, security, and compliance.

The Operations team, Security Committee, and Stratos AI Team regularly review and update tool categorisations. To request a new AI tool, submit details to your manager and the Operations Manager for review.

Please use tools thoughtfully in accordance with the tier that they have been assigned. When using tools, the following always apply:

- Use Stratos-provided accounts for company tools.
- Input only the minimum necessary data for the task.
- Adhere to Stratos' existing Data Protection and Security Policy, Privacy Policy, Personal Device Policy and Remote Working Policy.

Remember that we have little control over how data is stored by third parties, and as such when using an AI tool, you must be thoughtful about the data you provide and the potential impact if leaked.

Agents

Some AI tools offer "agent" functionality in which an AI agent will act on your behalf to perform tasks in its own "virtual" computer. You should not enter your credentials for the agent to use unless explicitly permitted by this policy or noted in the "Stratos Approved AI Software" document.

For any questions or clarifications regarding this policy, please contact your manager or Operations Manager.

Data Protection and Security Policy

Intent

Our clients trust us with access to their sensitive data, and we have a responsibility to protect this data. To fulfil this responsibility, we must apply thorough security controls to the way we work. Wherever possible Stratos will enforce system policies for compliance, however there is also a level of personal responsibility required to ensure our client's data is being protected and as such it is important you familiarise yourself with this policy.

Security Responsibilities

Stratos Director

- Provide guidance on incident response to Stratos staff.

Operations Manager

- Manage the implementation of new security initiatives.
- Ensure Stratos platforms comply with security best-practice.

Security Committee

- Pro-actively provide guidance to Stratos staff on security-related matters, including vulnerabilities and security-related initiatives.
- Review approval requests for new applications requiring access to Stratos data.
- Help to foster a security-aware culture within Stratos.

Stratos Staff and Contractors

- Ensure compliance with Stratos security and privacy-related company policies.
- Take responsibility for home networks used to access Stratos remotely to follow best practice advice from the Security Committee and compliance with Stratos BYOD and Remote Working policies.
- Take responsibility in verifying that requests for changes are legitimate. Ensure your passwords are not reused across multiple accounts, including any personal or work accounts.
- Report any security-related concerns you encounter to the Operations Manager.
- Update any software on their devices that they have installed outside of Stratos-managed software.

Security Breaches

If you have detected a security or privacy breach, contact a company director as soon as possible to take advice on next steps. It is important this takes place before any communication with potentially impacted clients as there may be insurance / contractual obligations which need to be considered.

If you have detected a breach of this policy then contact the Operations Manager to advise as soon as possible.

For more information on response processes, consult the Stratos Incident Response Plan.

Physical Controls

- Stratos uses Microsoft Azure datacentres in addition to AWS and Linode to host systems and access applications such as Microsoft 365. These datacentres are ISO 27001 compliant and include physical security measures including CCTV, swipe card access and security guards.
- Access to the Stratos office is protected using individually issued swipe cards.

Security Awareness

- All new staff and contractors must complete an online security and privacy training program as part of their onboarding process.
- All Stratos email accounts are subject to monthly phishing-awareness simulations with remediation training employed as required.
- All staff are briefed on relevant security-related matters on a fortnightly basis as part of a standing agenda item at the all-hands company meeting: The Wall.

Access Control – Stratos-Managed Systems

- All accounts across all systems must have MFA (Multi-factor Authentication) enabled. If you are using a Stratos account that does not have MFA enabled then please advise the Operations Manager as soon as possible. Always favour app-based authenticators over email or SMS.
- Where possible, access to client databases and applications will be restricted to the teams that are authorised to work on them. In some cases, where team members have moved teams, temporary access will be granted in order to maintain continuity of service during the transition period.
- All Entra ID accounts require Multi-factor Authentication. The available Multi-factor Authentication methods are restricted to the

Microsoft Authenticator App, OTP App, or FIDO-2 compliant security key. Push notifications via the Microsoft Authenticator App require number-matching to be completed.

- Where your Entra ID account is invited into an external tenant (including “Production” and “Development” Azure tenants) you must configure Microsoft Authenticator App, OTP App, or FIDO-2 compliant security key Multi-factor Authentication.
- Each staff member must ensure they use an individual, identifiable Entra ID account with a password that has not been re-used across any other accounts, nor has been used for personal accounts.
- Global Administrators in the STP Azure Tenant, NinjaRMM and IPassword have unique Administrator accounts, separate from their everyday user accounts.
- Privileged Identity Management is utilised in the STP Azure Tenant to elevate authorised group’s permissions on-demand for a period of up to 8 hours. Elevation attempts are audited and alerted upon.
- Entra ID and Stratos VPN access is geo-locked to New Zealand and Australia. Exclusion groups are available for staff to provide temporary access when travelling overseas. These groups are specific to the countries being visited.
- Clients should not be provided direct access to shared databases or storage objects within Stratos owned Azure tenants.
- Client data should only be accessed from Stratos-managed devices, or devices which comply with the security requirements of Stratos’s BYOD policy.
- Mobile Devices accessing the STP tenant are subject to Mobile Application Management policies, including encryption requirements, and prohibiting copy / paste operations with non-managed applications.

Access Control – Client-Managed Systems

- Stratos’s preferred position is for clients to provide individual, named accounts with MFA enabled, for each Stratos staff member requiring access to client-managed systems. In addition to this it is preferable that a client’s system only responds to requests from Stratos’s trusted IP address range (blue.stp.co.nz).
- A password should be set that has not been re-used across any other accounts

(including for Stratos internal systems), nor has been used for personal accounts.

- MFA should be enabled even when a client does not require it.
- When a Stratos employee or contractor leaves the company, as part of our defined offboarding checklist, Stratos will ensure the client is notified in advance so any accounts in the client-managed system(s) can be disabled.
- Stratos staff will connect from these trusted IP addresses from Stratos premises, or when connected remotely via the Stratos VPN.
- Credentials for client systems must only be stored within Stratos's password management platforms.
- Personal information is only accessed when required to fulfil a specific service for a client.

Data Storage

- All physical Stratos devices and BYOD devices must be encrypted:
 - Bitlocker (for Windows Devices)
 - FileVault (for MacOS Devices)
 - LUKS (for Linux Devices)
 - Pin enabled (for iOS devices)
 - Android encryption (for Android devices)
- Virtual Machines should be encrypted at the hypervisor or host Operating System layer. USB and other removable storage that contains client or Stratos data in which there would be an expectation of sensitivity must be encrypted using Bitlocker to Go (for Windows and Linux Devices) or FileVault (for MacOS Devices).
- Client databases should only be used for local development if the data has been obfuscated of all sensitive information first. In cases where data cannot be obfuscated in order to reproduce an issue, that data must only be stored on encrypted storage and must be removed immediately after testing.
- Devices which have been used to store client information must be securely destroyed and certified as such when they are retired.
- You must only store client data in approved locations.
- A list of approved locations can be found here: <https://stpznz.atlassian.net/wiki/spaces/Stratos/pages/158105601/Stratos+Approved+Client+Data+Storage+Locations>

- After deletion data will continue to be stored via Retention Policies and Backup storage until these policies allow the backups to expire. If you require data to be removed before this time then you can request this from the Ops Manager.

Data Transfer

- All data transferred must be encrypted in transit.
- Transfer of data must be via an approved method:
 - Passwords should only be transferred using the password sharing functionality of IPassword.
 - Client data should only be transferred using the following mechanisms:
 - SFTP
 - FTPS

If a client firewall does not support restricting by domain names, then you can find an up-to-date list of IP address using "nslookup blue.stp.co.nz"

- SCP
- CIFS/SMB (over an encrypted VPN tunnel)
- Remote Desktop Drive Redirection – Copy / Paste
- Encrypted USB storage device (within office only, wiped as soon as transfer complete)
- Client-approved file transfer mechanism
- Securely shared from the stp.co.nz SharePoint / OneDrive tenant.

Backup and Recovery

- Stratos Microsoft 365 tenant is backed up by 3rd party backup services, 4 times per day, with backups retained indefinitely.
- Data stored in SharePoint Online, Exchange Online, OneDrive and Teams are subject to Microsoft 365 Retention policies.
- Stratos Azure Virtual Machines must be backed up using Azure Backup, with a minimum backup frequency of once per day, with daily restore points retained for 7 days, weekly restore points retained for 5 weeks, and monthly restore points retained for 6 months.
- Stratos Virtual Machines hosted elsewhere must be backed up using equivalent backup, with a minimum backup frequency of once per day, with daily restore points retained for 7 days, weekly restore points retained for 5 weeks, and monthly restore points retained for 6 months.

- All backups should be configured with Backup heartbeating via BackupRadar to ensure successful backups.
- Test restores for any backed up data should be performed on a monthly basis. The restore process should be documented in the appropriate wiki for each backed up system. Backup data should be encrypted at rest.

Vulnerability Mitigation

- All Stratos-managed and BYOD devices should have security updates applied within 7 days of release. Critical updates should be applied as soon as possible.
 - Stratos-managed Windows and Linux desktops are scanned for missing Operating System Updates and pre-defined 3rd party software updates on a daily basis using 3rd party patch management software.
 - Stratos-managed Mac desktops have declarative updates issued via device management policy.
 - Stratos-managed Windows desktops have outstanding Windows Operating System patches installed on a weekly basis using 3rd party patch management software.
 - Stratos-managed Windows and Mac desktops have outstanding pre-defined 3rd party software updates installed on a daily basis using pre-defined 3rd party patch management software.
- Stratos-managed Firewalls have security updates installed within 3 days of release. • Stratos-managed Firewalls utilise Advanced Threat Protection technologies such as IPS to block detected exploitation of vulnerabilities.
- Stratos Security Committee convenes on a weekly basis to review vulnerability advisories from CERT-NZ and CISA and advises Stratos on impacted services requiring remediation.

Email Protection

- All inbound email to stp.co.nz, stp.nz, alchemy.co.nz and alchemy.nz is processed by Microsoft Exchange Online Protection including:
 - Safe Attachments processing to detonate attachments in a sandbox before delivery.
 - Safe Links URL re-writing.
 - Anti-phishing with Impersonation Protection enabled.
- Anti-spam and anti-malware strict policies.

- Stratos email domains are protected by DMARC policies, which are set to Reject. Stratos email systems utilise DKIM and SPF protection.
- The Operations Manager receives and reviews weekly DMARC policy compliance reports.

Anti-Virus Protection

- All Stratos-managed and BYOD devices with the exception of iOS devices², must have active and up to-date antivirus software installed with real-time scanning enabled.
 - All Stratos-managed Windows and Mac devices have Microsoft Defender deployed and configured via policy.
 - In addition, all Stratos-managed Windows and Mac devices have Huntress EDR deployed via policy.
 - All Linux devices should have Clam AV enabled for antivirus services.
- Stratos-managed firewalls also scan and intercept traffic using anti-virus profiles. • Stratos email is scanned via Microsoft Exchange Online Protection.

WiFi Security

- Staff should limit use of public or unsecured WiFi networks to connect to Stratos or Stratos client services, instead Stratos staff should utilise Personal Hotspot functionality on their mobile device. If this is not possible, then connecting to the Stratos VPN whilst connected to a public hotspot will reduce the security risk. The VPN must NOT be set to ignore certificate errors.
 - If staff receive a Certificate Error when attempting to connect to a public WiFi or the Stratos VPN then disconnect from the WiFi immediately.
 - Stratos will re-imburse staff for any reasonable costs that this may incur from Personal Hotspot utilisation.
- Stratos Office Staff WiFi is encrypted using strong encryption.
- Individual WiFi keys are assigned to each device as it is provisioned.
- When a device is retired the WiFi key for the device is revoked.
- When a staff member leaves, the WiFi keys for their devices are rotated.
- Stratos maintain a separate, firewalled Visitors WiFi network.
 - The Visitors WiFi network does not use a Internet

IP address from Stratos's trusted IP address range (blue.stp.co.nz).

- The Visitors WiFi network does not have access to any other Stratos internal networks. o The Visitors WiFi network employs host isolation.

Monitoring

- Stratos-managed Windows devices and office firewall have logs sent to a central SIEM service. All Stratos-managed backup services must be configured to send backup reports to Stratos's BackupRadar heartbeat service.
- Stratos-managed Windows devices have active alerting on specific security events recorded on each device's Security event log including:
 - Failed logon thresholds
 - Windows Security event log full
 - Windows Security event log cleared
 - Windows Defender service stopped
 - Bitlocker encryption disabled

Password Sharing

- Passwords should be shared with 3rd parties using the IPassword sharing mechanism only.

² Apple does not currently permit anti-virus software on the iOS platform. This policy will be updated to include iOS when Apple's policy changes.

- When passwords are received from 3rd parties they should (where possible) be immediately self reset and stored in IPassword.

Verifying Identity

- Stratos staff may receive requests to grant access or additional permissions to client resources. Stratos staff are responsible for verifying the request as authorised and legitimate: o The staff member placing the request is authorised to do so by the client.
 - The identity of the requestor has been verified as legitimate.
 - Verification should be performed by placing a voice call to the requestor. The phone number of the requestor should be confirmed via Accelo or the company's web site (Not from the email request itself).

Staff Onboarding

- Prospective staff undergo background checks including a police check before they are employed. The induction process covers privacy and security responsibilities.

- The induction process includes cyber security training.

Staff Offboarding

- Staff and contractors leaving Stratos have their Stratos accounts disabled on the day that they leave employment.
- Any clients that they had access to are advised no later than the last day of employment, in order to allow clients to facilitate disabling accounts on their platforms.
- Credentials the staff member has accessed in the last 12 months are rotated as part of the offboarding process.
- The staff member's unique WiFi key is rotated on the day that they leave employment.

Privacy Policy

Intent

At Stratos we believe in the right to privacy for our employees, contractors and clients. This policy helps to set some expectations and guidelines around Stratos's commitment to privacy.

Awareness of Privacy legislation

- Stratos expects that you are familiar with your responsibilities required by the Privacy Act (2020), the Health Information Privacy Code (when working with clients in the health sector), and the Australian Privacy Act (1988) (when working with Australian-based clients).
- To facilitate this awareness, Stratos encourages you to complete the Privacy ABCs and Health Privacy ABCs from the New Zealand Privacy Commission and Privacy in Practice from the Office of the Australian Information Commissioner. You can find links to complete these in the Employment Hero certifications section.

Privacy Officer

- The role of the Privacy Officer at Stratos is fulfilled by the Operations Manager.
- If you have any questions about this policy, lodging complaints, using the rights to access and correct personal information or any other privacy matter, please contact the Privacy Officer.

Workplace Presence

- Whilst Stratos has a flexible working from home policy, there is also a responsibility to quickly ensure all staff in the office are accounted for in the event of an emergency.

- To assist with this, you are encouraged to sign-in to and out of the office using the systems provided.
- Data collected in this system will solely be used for the purpose of accounting for staff in the event of an emergency, or an equivalent test or drill, or for contact tracing purposes.

Computer Monitoring

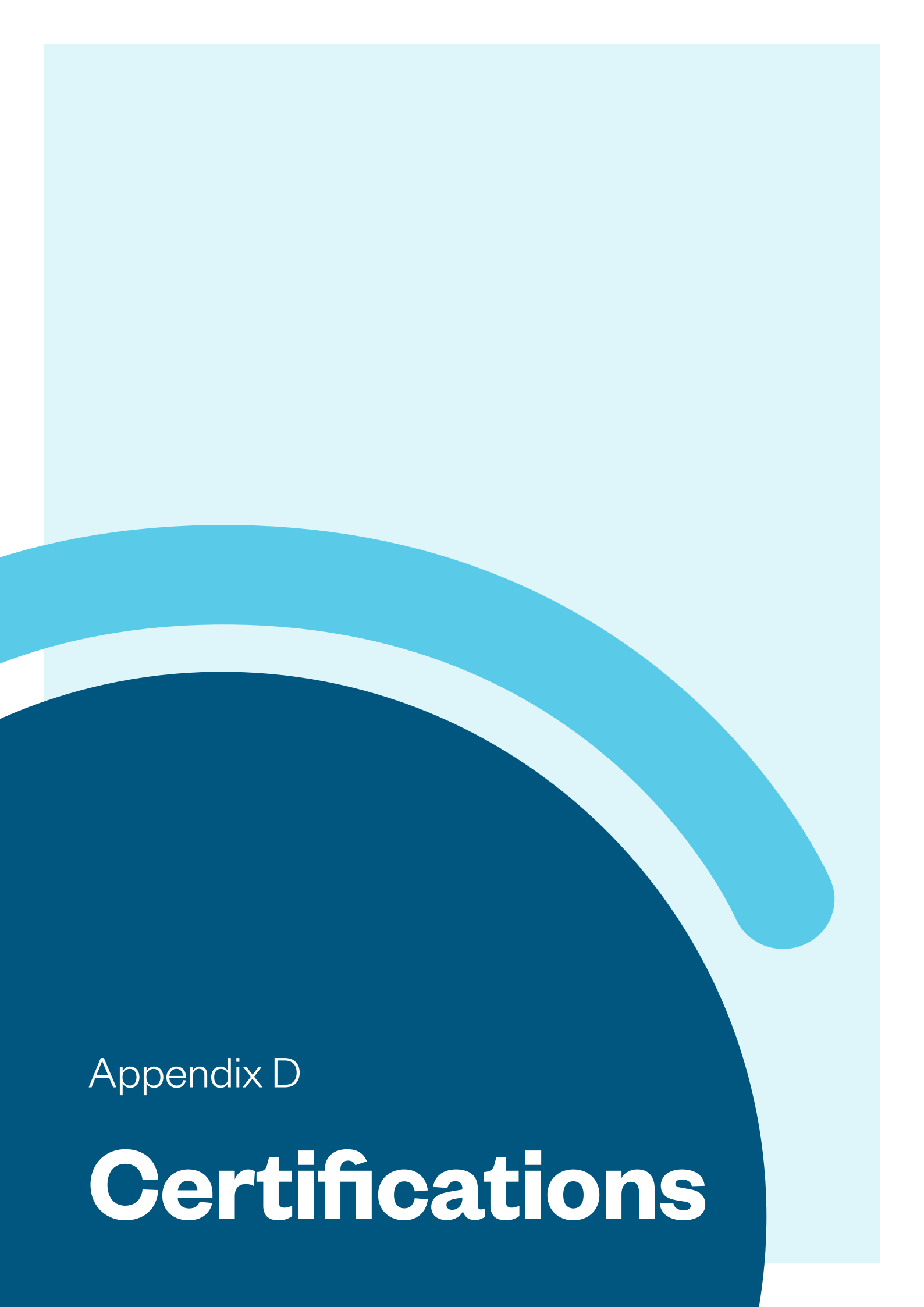
- Stratos may deploy monitoring and management software on company-owned computers for the purposes of:
 - Ensuring software is up to date, particularly in relation to security updates
 - Deploying new software
 - Ensuring the computer meets minimum security requirements
 - Asset management
 - Alerting on hardware and software faults.
- Stratos may require Microsoft Intune enrolment on personally owned computers used to access Stratos or Client data for the purposes of ensuring the device meets Stratos minimum security requirements.
 - You can find more information on the privacy implications of enrolling your device in Intune here: <https://learn.microsoft.com/en-us/mem/intune/user-help/what-info-can-your-company-see-when-you-enroll-your-device-in-intune>
 - Where you are not comfortable enrolling your personally owned computer in Microsoft Intune, Stratos can provide you a Virtual Machine to operate in a sandbox on your device.
- Stratos will not use monitoring or management software for the purposes of performance management of staff.
- Stratos will regularly audit monitoring and management software to ensure it is only being utilised in line with the intent specified in this policy.

Communications Platforms

- Stratos operates a number of communications platforms enabling collaboration between teams and clients.
- Meeting attendees (both online and in-person) must be notified of voice or video recording by the person making the recording.
- For the purposes of data protection, Stratos systems may have compliance policies in place to retain deleted data, including records of public and private communication.

Employee Records

- Stratos may collect and store personal information that is directly related to your employment with us including:
 - Your name and contact details
 - Your bank details, Kiwisaver and tax information
 - Your emergency contacts
 - Qualifications and training records
 - Criminal background checks
 - Eligibility to work
 - Your curriculum vitae
 - Your employment contract
 - Performance review responses
 - Documentation of HR matters
 - Other information directly related to your employment with Stratos.
- This information may be hosted on 3rd party systems, including systems located outside of New Zealand.
- We will retain your personal information for up to 6 months after the termination of employment with Stratos. After this time we will undertake reasonable endeavours to remove your information from our systems.
- The following staff may have access to your personal information stored within our HR system: Board of Directors, CEO, Team Leaders and Managers, Operations Manager, Office Manager.
- Your personal information will only be made available to 3rd parties where required by law, or where disclosure is necessary to avoid endangering someone's health or safety.
- Subject to some exceptions under privacy laws, you have a right to see and have a copy of your personal information and advise of any changes necessary if the information is not correct. Where we decide not to make a requested correction to your personal information and you disagree, you may ask us to make a note of your requested correction with the information.
- Please reach out to your manager to obtain a copy of, or issue a correction to the personal information Stratos stores.



Appendix D

Certifications

Stratos Training Certifications

Name	Qualifications	Issue Date
James Houden	AZ900 – Microsoft Certified: Azure Fundamentals	29/04/2026
Tom Lightfoot	AZ900 – Microsoft Certified: Azure Fundamentals	20/05/2025
Blake Burgess	AZ400 – DevOps Engineer Expert	23/09/2021
Blake Burgess	AZ204 – Microsoft Azure Developer Associate	22/12/2020
Yerren van Sint Annaland	AZ900 – Microsoft Certified: Azure Fundamentals	27/08/2025
Yerren van Sint Annaland	AI900 – Microsoft Certified – Azure AI Fundamentals	09/12/2025
Agnes Park	AI900 – Microsoft Certified – Azure AI Fundamentals	23/03/2026
Agnes Park	AZ900 – Microsoft Certified: Azure Fundamentals	02/03/2025
James Gardner	AZ204 – Microsoft Azure Developer Associate	13/06/2023
Josh Lloyd	AZ204 – Microsoft Azure Developer Associate	30/11/2023
Kaveshan Karunakaram	AZ204 – Microsoft Azure Developer Associate	24/06/2026
Ash Lightfoot	PL200 – Power Platform Functional Consultant Associate	18/10/2023
Tyler Price	AZ104 – Microsoft Azure Administrator Associate	15/12/2021
Nik Binzenbach	PL900 – Microsoft Certified: Power Platform Fundamentals	01/08/2025
Divya Srinivasa Raghavan	PL900 – Microsoft Certified: Power Platform Fundamentals	04/12/2022
Tyler Price	AZ-305 – Microsoft Azure Solutions Architect Expert	13/05/2025
Ash Lightfoot	PL400 – Microsoft Certified: Power Platform Developer Associate	10/12/2025
Divya Srinivasa Raghavan	PL200 – Power Platform Functional Consultant Associate	18/12/2025
Tyler Price	PL900 – Microsoft Certified: Power Platform Fundamentals	02/04/2026
Asma Lulu	AZ900 – Microsoft Certified: Azure Fundamentals	05/12/2021
Asma Lulu	AI900 – Microsoft Certified – Azure AI Fundamentals	08/12/2021
Asma Lulu	PL900 – Microsoft Certified: Power Platform Fundamentals	11/12/2021
Asma Lulu	PL400 – Microsoft Certified: Power Platform Developer Associate	07/02/2025
Asma Lulu	DP600 – Microsoft Certified: Fabric Analytics Engineer Associate	07/04/2025
Dhugald Spry	MS900 – Microsoft 365 Certified: Fundamentals	29/01/2025
Nicole Tulett	MS900 – Microsoft 365 Certified: Fundamentals	21/03/2025
Polina Babich	AZ900 – Microsoft Certified: Azure Fundamentals	15/07/2026

